2nd MBBS Examination

Faculty of Medicine – University of Ruhuna

Anatomy

Classified MCQ Questions with Reviews

Upper Limb

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PREFACE

Every medical student knows the weight of the 2nd MBBS Examination, it is a challenge that demands more than just knowledge. For 45th Batch us , this exam became a shared journey, an experience that pushed us beyond our boundaries.

So that , for supporting our juniors, we've rearranged this MCQ collection by adding past 3 batches questions. So this include 43rd batch to 45th Batch 2nd MBBS and Term MCQs and SBRs with review. There may be doubts with some answers because they were made by us, not professionals :)

We hope this one will help bridge the gap between where you are and where you aim to be. And special thank to 42nd batch for continue this MCQ collection upto thier batch. We wish you all success in your journey to becoming the healers the world needs.

"The real test is not to avoid mistakes, but to learn from them and keep moving forward."

#WeAre45th

ACKNOWLEDGEMENTS

We would like to express our sincere thanks to 38th Batch and 42nd Batch, who dedicated their valuable time to classify the Past Paper MCQs which enabled us to create this review.

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Surface Anatomy of Upper Limb

1) **T/F regarding the surface anatomy of upper limb**
   a) Clavicle is subcutaneous throughout its length.
   b) Coracoid process of scapula can be palpated immediately beneath the middle and outer 1/3 of clavicle.
   c) The ulna can be palpated along its anterior length.
   d) Annular ligament surrounded the neck of the radius.
   e) Growing end of radius is upper end.

2) **WOF is very difficult to palpate in living subject**
   a) Superior angle of scapula.
   b) Spine of scapula.
   c) Coracoid process under the junction between middle and lateral 1/3 of clavicle.
   d) First rib
   e) Inferior angle of scapula

3) **T/F regarding the surface anatomy**
   a) Head of the humerus can be felt in the axilla.
   b) The coracoid process can be felt just lateral on the infraclavicular fossa.
   c) The head of the radius can't be palpated in the elbow.
   d) Subclavian artery pulsation can be felt against the 1st rib.
   e) The distal wrist crease indicates proximal margin of the flexor retinaculum.

4) **WOF nerves are palpable**
   a) Supraclavicular nerves
   b) Radial nerve
   c) Median nerve
   d) Musculocutaneous nerve
   e) Ulnar nerve

5) **T/F regarding the surface anatomy**
   a) Lateral border of the scapula can be both seen and felt.
   b) Coracoid process of the scapula lies immediately below the clavicle at the junction between medial 2/3 and lateral 1/3 of the clavicle.
   c) The head of the radius can be felt during supination and pronation over the hollow in the posterolateral aspect of the extended elbow.
   d) The dorsal tubercle of Lister is palpable on the anterior aspect of the distal end of the radius.
   e) Pisiform can be palpated at the base of the hypothenar eminence.

6) Regarding the surface anatomy of upper limb

a) Recurrent branch of the median nerve lies about one finger breadth distal to the tubercle of the scaphoid.
b) The hook of the hamate bone can be felt on deep palpation of the thenar eminence.
c) The ulnar nerve at the wrist lies immediately lateral to the ulnar pulse.
d) The styloid process of the radius lies in the floor of the anatomical snuff box.
e) Axillary nerve is closely related to the anatomical neck of the humerus.

7) True or false regarding the surface anatomy of the bones of the upper limb

a) Clavicle is subcutaneous throughout its length
b) Humeral head can be palpated in the axilla when the arm is adducted
c) Scaphoid bone can be felt in the anatomical snuff box
d) Lesser tubercle of humerus is the most lateral bony point of shoulder
e) The triangular relationship between olecranon process of ulna and epicondyles of humerus is disturbed in supracondylar fracture.

8) True or false regarding the bony landmarks of the upper limb

a) Clavicle is palpable throughout its length.
b) Greater tubercle of humerus is on the summit of the shoulder.
c) Olecranon process and two epicondyles form a tringle at the elbow.
d) Radial styloid process is palpated distal to ulnar styloid.
e) Scaphoid lies in the floor of anatomical snuff box.

Bones of the Upper limb

9) True or false

a) Posterior border of the ulna is subcutaneous.
b) The pisiform is felt at the base of the thenar eminence.
c) Lower part of the humerus engaged forward at the distal end of the forearm.
d) The capitates is the first bone to ossify in the carpus.
e) Nutrient artery of the radius directs towards the elbow joint.

10) True or false

a) The capsular ligament of the shoulder joint is attached to the whole of anatomical neck of the humerus.
b) Knuckles are formed by bases of phalanges.
c) In resting position the glenoid cavity is directed anteriorly and laterally.
d) Contraction of deltoid would draw the head of the humerus against the coracoacromial arch.
e) Lateral and medial epicondyles of the humerus and the olecranon process lie in the same line when the forearm is extended.

11) T/F regarding the bones of the upper limb

a) Lunate can dislocate anterior.
b) Ulna has its growing end of its proximal end.
c) Fracture of the scaphoid can be identified clinically by the tenderness over anatomical snuff box.
d) Scaphoid fracture can be diagnosed early by using X-ray.
e) Coracoids process is an atavistic epiphysis.

12) T/F regarding bones

a) Carpal bones are ossified by a single ossification center.
b) In a lateral X-ray, loss of reduction of forward angulation of lower end of humerus indicates a fracture of lower 1/3 of humerus.
c) Epiphysis of the 1st metacarpal is placed at its distal end.
d) Upper end of radial epiphysis fuse with the shaft of radius later than that of the lower end.
e) The tip of the styloid process of the radius is placed proximal to that of the ulna.

13) Regarding fractures of bones of the upper limb

a) Surgical neck fracture is the commonest fracture.
b) When the radius is fractured proximal to the insertion of pronator teres, the proximal part becomes supinated.
c) In Colle's fracture the distal segment is displaced posteriorly & superiorly.
d) Fractures of the scaphoid can be identified by an immediate X-ray.
e) Dislocation of the lunate is a common cause of the carpal tunnel syndrome

Clavicle

14) Regarding the clavicle

a) First bone to ossifying.
b) Has no medullary cavity.
c) Most commonly fractured bone in the body.
d) Has a cartilaginous development.
e) Develop in the 6th week of intrauterine life.

15) Regarding the clavicle

a) Bony prominence over the shoulder is formed by the lateral end of the bone.
b) The cords of the brachial plexus lie behind the middle end of the bone.
c) Ossify in membrane.
d) When fractured the lateral part is elevated by trapezius.
e) Always has an epiphysis of its medial end.

16) T/F regarding the clavicle

a) Is palpable throughout its length.
b) Has a medullary cavity.

c) Trunks of brachial plexus pass behind its lateral 3rd part.
d) Subclavian vessels are at risk in fracture of clavicle.
e) The ossification initiate while in the uterus.

17) Regarding clavicle

a) Clavicle has a medullary cavity.
b) Clavipectoral fascia lies in the anterior axillary wall.
c) Clavicle has an attachment to the 1st rib.
d) The clavicle is palpable throughout its length.
e) Coracoid process is covered by deltoid muscle.

18) The clavicle

a) Red bone marrow is present.
b) It has no secondary centers of ossification.
c) Its forward movement is produced by trapezius.
d) Coracoclavicular ligament attached to inferior surface.
e) It transmits weight of upper limb to axial skeleton.

Scapula

19) The scapula

a) Covers parts of the 2nd to 7th ribs.
b) Cannot move at the acromioclavicular joint.
c) Have muscular attachments to the vertebrae, skull and ribs.
d) Forms a boundary of auscultation.
e) Has lower 5 digits of serratus anterior attached to its medial margin.

20) True or false regarding scapular region

a) Inferior angle is at the level of spine of 7th thoracic vertebra
b) Suprascapular artery contributes to scapular anastomosis
c) Supraspinatus is attached to greater tubercle of the humerus
d) Teres minor is a medial rotator of arm
e) Injury to long thoracic nerve produces winged scapula

21) Regarding the scapula

a) Its spine is at the level of T_3.
b) Coracoid process can be depressed by the anterior fibers of deltoid.
c) The tendon of supraspinatus passes behind the coracoacromial arch.
d) The epiphysis of the scapula is jointed to the body by puberty.
e) The subscapular artery does not take part in scapular anastomosis.

22) Regarding the scapula

a) Trapezius and serratus anterior are the lateral rotators of scapula.
b) In coarctation of aorta the blood supply to lower part of the body is stopped.
c) A bursa lies anterior to the neck of the scapula.
d) Coracoid process of the scapula ossifies separately by 2 centers and joints with the rest of the scapula at 15 years.
e) Groove for the circumflex scapular artery lies between the attachments of teres major and minor muscles.

23) Scapula has a

a) Coracoids process gives attachments to the long head of triceps.
b) Glenoid cavity below which the long head of triceps is attached.
c) Costal surface dividing by a projecting spine into supraspinous and infraspinous fossae.
d) Palpable inferior angle which overlies the 7th rib.
e) Lateral border that attaches to serratus anterior.

24) Regarding the scapula

a) Has a coracoids process projecting directly forwards above the clavicle.
b) Is rotated on chest wall by pectoralis minor so that the inferior angle moves laterally.
c) Ossifies in membrane.
d) Has the tendon of long head of biceps brachi muscle attached to its infraglenoid tubercle.
e) As it moves upwards on the chest wall is associated with an upward movement of the medial end of clavicle.

25) True or false regarding the scapula

a) Pectoris minor insert into the tip of coracoid process
b) Trapezius helps in lateral rotation
c) Supraglenoid tubercle gives attachment to the long head of triceps
d) Thoracodorsal artery is closely related to its medial border
e) Its medial boarder forms the lateral boarder of the auscultatory triangle

26) True /False regarding scapula

a) Serratus anterior protracts the scapula
b) The medial end of the clavicle moves backward during the retraction of scapula
c) It has 7 primary centers
d) Posterior interosseous arteries contribute to the anastomosis around the scapula
e) Damage to the thoracodorsal nerve causes winging of scapula

27) True or false regarding the muscle of the scapular region,

a) Subscapular muscle inserts to lesser tubercle of humerus
b) Teres minor rotate arm laterally at shoulder joint
c) Subscapularis is medial rotator of the shoulder
d) Paralaysis of the serratus anterior leads to winging of the scapula
e) Rhomboid muscles are supplied by the suprascapular nerve

Humerus

28) Regarding humerus

a) Anatomical neck is related to the axillary nerve.
b) Has a nutrient foramen directed into downward.
c) Has the radial nerve posterior to it.
d) Brachialis muscle is anteriorly attached to the humerus.
e) Adult's humerus has red marrow.

29) T/F regarding humerus

a) In anatomical position head of the humerus faces anteromedially.
b) The grater tubercle forms the most lateral bony part of the shoulder.
c) Supracondylar lines are palpable.
d) Lower end is the growing end.
e) Medial epicondyle is extracapsular.

30) Movements affected when entire head of the humerus is broken

a) Abduction
b) Lateral rotation
c) Flexion
d) Extension
e) Adduction

31) Regarding humerus

a) Has part of the supinator muscle attached to the lower end.
b) The lower end has the ulnar nerve running posterior to the lateral part.
c) Has a nutrient foramen directed downwards.
d) In adults contain red marrow.
e) Has the subscapularis muscle attached to the greater tuberosity.

32) T/F regarding the humerus

a) Epiphysis of lower end fuses before than epiphysis of upper end
b) Radial nerve is damaged in a fracture of the surgical neck
c) Nutrient artery is derived from the profunda brachii artery
d) Capsule attached to the anatomical neck

Radius

33) Regarding the radius

a) Annular ligament surrounds the neck of the radius.
b) Lateral part of inferior articular surface is in contact with trapezium.
c) Cannot be palpated.
d) Brachialis is attached to the tubercle.

e) The articular disk of the inferior radio-ulnar joint is attached to the lower margin of the ulnar notch.

34) The radius

a) Possesses a head which articulates with the scaphoid and lunate.
b) Gives attachment to the triceps tendon.
c) Possesses a palpable styloid process.
d) Is attached to the ulna throughout the length of its interosseous border.

35) Regarding the radius

a) Has the tendon of biceps muscle attached to the whole of the tuberosity.
b) The tuberosity can be felt on the posterior surface of the forearm.
c) Head of the radius can be palpated in the depression of the extended forearm at the back of the elbow.
d) The joint between the upper parts of the radius and ulnar is a hinge joint.
e) The pronator teres is attached to the region of maximum convexity.

36) Fracture at the head of the radius

a) Is possible with fracture of the shaft of the humerus.
b) Will impair the supination of the forearm.
c) Loss of sensation of all over the dorsal surface of thumb.
d) Does not affect the ability to grip.
e) Can't flex the interphalangeal joint of the thumb.

Ulna

37) Regarding the ulna

a) The flexor digitorum profundus muscle attached to its anterior and medial surfaces.
b) Has the triceps muscle attached to the posterior surface of the olecranon.
c) Has a direct distal articulation with the triquetrum bone.
d) Extensor carpi ulnaris tendon lies in the groove at posterior surface of the lower end.
e) The apex of the articular disk of inferior radio-ulnar joint is attached immediately medial to styloid process.

38) The ulna

a) Gives attachment to the brachialis muscle.
b) Possesses a styloid process on the anteromedial surface of its lower end.
c) Articulates at its lower end with an articular disc.
d) Is palpable over the whole length of its posterior border.
e) Is related inferiorly to the extensor carpi radialis muscle.

Carpal bones

39) Regarding the carpal bones

a) Pisiform is a sesamoid bone.
b) There anterior and posterior surfaces are rough for attachment of ligaments.
c) Fracture of the capitates produces tenderness in anatomical snuff box.
d) The carpal bones are arranged in such a way to convex anteriorly.
e) The trapezium has saddle shape articular surface.

40) The carpal bones

a) Are arranged into proximal, middle and distal rows.
b) Which form the distal articular surface of the wrist joint are the scaphoid, lunate and pisiform.
c) Give attachment to the flexor & extensor retinaculum.
d) Give attachment to the lumbrical muscles.

Ossification of bones of the upper limb

41) True or False

a) There are seven secondary ossification centers in humerus.
b) In long bone ossification it extends along the body of the bones as endochondral ossification.
c) Proportionate of the face of a new born baby to the skull is greater than in an adult.
d) Ossification of the capitate is finished in 4th year.
e) Coracoclavicular ligament is an example for syndesmosis

42) T/F regarding the ossification of the upper limb bones

a) Ossification centers will appear in humerus at 6th month of intra uterine life.
b) An example for saddle joint is carpometacarpal joint of the thumb.
c) At 2 years, the upper end of humerus has 2 secondary centers.
d) A sound knowledge on ossification of bone is very useful to a medico legal expert.
e) Epiphysis of growing ends of radius and ulna fuse with their shaft around 20 years.

43) On the upper limb the secondary epiphysis are formed

a) At both ends of the radius and ulna.
b) At the bases of all the phalanges.
c) At the distal ends of all the metacarpals.
d) At the tuberosity of the radius.
e) At the carpal bones.

44) Regarding ossification

a) Ossification centre for scaphoid appears at the 12th year.
b) In a child of 7 years epiphysis of the ulna has separated but not the lower.
c) At 20 years all the epiphysis of the scapula has joined the body.
d) Lower end of humerus has its four epiphyses by 12 years.
e) The pisiform ossifies at the age of 12 years.

45) True or false

a) The lower end of humerus fuses with the shaft before the upper end.
b) In radius the secondary centre of ossification is present at birth.
c) Both radius and ulna have their growing ends proximally.
d) The ossification centre of the styloid process fuses with the secondary centre of the radius at 18 years.
e) The lower part of olecranon is ossified by an upward extension of the ossification of the shaft.

Pectoral Region

46) T/F regarding the pectoral region

a) Pectoralis major muscle is supplied by both medial and lateral pectoral nerves.
b) Clavipectoral fascia is pierced by both medial and lateral pectoral nerves.
c) Pectoralis muscle is powerful medial rotator of shoulder joint.
d) Coracobrachialis is pierced by median nerve.
e) Radial nerve winds around the axillary artery.

47) T/F about the pectoral region

a) The clavipectoral fascia lies in the anterior wall of the axilla deep to pectoralis major.
b) Lateral pectoral nerve enters the deep surface of pectoralis major inferior to the pectoralis minor.
c) Lymphatics pierce the clavipectoral fascia to enter the apical group of axillary lymph nodes.
d) Pectoralis major medially rotates the humerus.
e) Malignancies of the breast are common in the upper outer quadrant.

48) T/F regarding the pectoral region

a) Lateral pectoral nerve pierces the clavipectoral fascia.
b) Thoracodorsal artery is the continuation of lateral thoracic artery.
c) Lateral group of axillary lymph nodes are located along the lateral thoracic vessels.
d) Axillary vein is related to the 2nd part of axillary artery.
e) Glandular tissues are numerous in the upper outer quadrant of the breast.

49) The clavipectoral fascia

a) Is attached to the 1st rib.
b) Is pierced by the cephalic vein.
c) Is in the anterior axillary wall.
d) Continuous with axillary fascia.
e) Makes the floor of axilla.

50) Structures pierced by the clavipectoral facia include,

a) Median pectoral nerve
b) Intercostobrachial artery
c) Thoracoacromial artery
d) Cephalic Vein
e) Superficial lymph drainage of the upper limb

51) Which pierces the clavipectoral fascia?

a) Medial pectoral nerve
b) Lateral pectoral nerve
c) Pectoralis minor
d) Cephalic vein
e) Thoraco acromial artery

Mammary Gland

52) Regarding mammary gland

a) Developed from ectoderm.
b) Retro mammary space lies behind the deep fascia.
c) Carcinomas are more common in upper lateral quadrant.

d) Milk accumulates in lactiferous duct during lactation.
e) Parasternal lymph nodes are the commonest site of the breast cancer.

53) Regarding the female breast

a) It is in the superficial fascia.
b) It is a modified merocrine gland.
c) No hair in areola.
d) Mainly drain into anterior axillary nodes.
e) Dimpling due to retraction of cooper ligament.

54) T/F regarding mammary gland

a) Axillary tail passes in to the axilla at the level of 2nd rib.
b) The nipple is free of fat.
c) There is a communication of lymph vessels across the median plane.
d) Obstruction of the superficial lymph vessels give rise a pea d'orange appearance.
e) Venous drainage of the breast communicates with the vertebral venous plexus.

55) Female mammary gland

a) Is enclosed in deep fascia.
b) It is a modified sweat land.
c) Has lymphatic drainage to the anterior mediastinum.
d) Extend from the lateral side of the mid-axillary line.
e) It has a capsule.

56) Regarding the mammary gland

a) Lobes can be identified by palpation.
b) It is supplied by subclavian and axillary arteries.
c) Suspensory ligaments are attached to the skin.
d) Has its main lymphatic drainage to pectoral group of axillary lymph nodes.
e) Lies on pectoralis major and serratus anterior.

57) T/F regarding the breast

a) There are 15 – 20 lactiferous ducts.
b) Base extends from 2nd rib to 6th.
c) Each lobe is covered by a separate ligament.
d) The axillary tail extends up to 3rd rib.
e) The upper part of the breast is supplied by intercostal arteries.

58) Regarding the breast

a) The axillary tail lies in the superficial fascia.
b) Glandular tissues are numerous in the upper lateral quadrant.
c) It is a modified sweat gland.
d) Peau d'orange like appearance in breast carcinoma due to blockage of superficial lymph vessels.
e) Functional alveoli develop in early pregnancy.

59) Female mammary gland

a) It partly on the skin on the serratus anterior muscle.
b) The nipple lies between 3rd and 4th ribs.
c) Dimpling of the skin over the breast is an evidence of underlying carcinoma.
d) As abscess of the breast should be opened by a radial incision.
e) At the menopause the glandular tissues atrophy.

60) Regarding female breast

a) It develops from an investigation of chest wall ectoderm.
b) It is partly lie on serratus anterior.
c) Nipple and areola drain with superficial lymphatic's of the breast.
d) An abscess in a breast should be opened in a radial incision.
e) Recent retraction of the nipple is suggestive of breast carcinoma.

61) True or False

a) Has a sub mammary space which allows certain degree of mobility.
b) The majority of the breast cancers develop in superior lateral quadrant.
c) Retro mammary space lies deep to the deep fascia.
d) In male and pre-pubertal female nipple is at the level of 4th intercostal space.
e) The lateral quadrants of the breast drain into anterior group of axillary lymph nodes.

62) What is the best statement regarding the breast

a) Mammary gland is modified cutaneous gland.
b) Nipple inversion donates an underlying malignancy.
c) Breast tissue extend deep to the deep fascia.
d) The ligament of Cooper extends from subcutaneous tissue to the pectoral tissue.
e) Peau'd orange appearance over skin is due to skin infiltration of breast carcinoma.

63) True/false regarding mammary gland

a) It is a modified sebaceous gland
b) It overlies serratus anterior muscle
c) It is supplied by anterior intercostal artery
d) It is supplied by anterior intercostal artery
e) Lymph drain into the retrosternal lymph nodes

64) T/F regarding female breast

a) It extends vertically from 2nd to 6th ribs.
b) It is deep to pectoral fascia.
c) Lymph vessels from breast mainly drain into axillary lymph nodes.
d) Peau d'orange appearance of the carcinoma of the breast is due the tumor infiltration into the breast.
e) Nipple retraction in breast cancers is due to the involvement of Cooper's ligament.

65) The mammary gland

a) Breast is a specialized sebaceous gland.
b) Is attached to pectoral fascia.
c) Have Montgomery glands in areola.
d) Has posterior intercostal arterial supply.
e) Peau'd orange appearance occurs in malignancy obstructive superficial lymph vessels.

66) Regarding female breast

a) Increases in size during puberty mainly due to addition of adipose tissue.
b) Has lactiferous ducts which open by a single orifice on the nipple.
c) Areola contains smooth muscle fibers which cause the erection of the nipple.
d) Ducts and alveoli are ectodermal in origin.
e) Lymphatics communicate with the opposite breast across the midline.

67) Regarding the lymph drainage of the breast

a) Most of the lymph drains to the axillary nodes.
b) Middle part is drained to the parasternal nodes.
c) There is lymph passing through the pectoralis major.
d) There is communication with the other breast.
e) Peau'd orange is due to infiltration of lymphatics by cancer cells.

68) Female breast

a) Extend from the outer border of the sternum to the anterior axillary line.
b) Has an axillary tail passing superolaterally into the axilla.
c) Consist of 30 -35 lobes in lactating females.
d) It is divided by connective tissue septa in to lobes.
e) Lies entirely in superficial fascia.

69) T/F regarding female breast

a) Lies directly on external intercostal muscles.
b) Has its lymph passing through subareolar plexus before entering lymph nodes.
c) It is fully encapsulated.
d) Is movable over the pectoralis major when it contract.
e) Has its each lobe draining to a single lactiferous duct.

70) Lymph vessels of the breast drain into

a) Axillary nodes on same side
b) Lymph nodes on opposite side
c) Internal thoracic node
d) Internal mammary lymph node
e) Cervical nodes

71) Arterial supply to mammary gland is derived from

a) Internal thoracic artery
b) Intercostal arteries
c) Lateral thoracic artery
d) Thoracoacromial artey
e) Thoracodorsal artery

72) True and false about breast

a) Lactiferous sinus lies deep to areolar of breast
b) Glandular tissue are found easily in its upper quardrants
c) Suspensory ligaments are well developed in its lower quadrants
d) Paeu'd orange appearance in the breast is due to incompetent cooper ligaments
e) Floor is supplied by the intercostal brachial nerve

73) True or false regarding female breast

a) It is covered by pectoral fascia.
b) Lobes are separated by interlobular septa.
c) Is supplied by internal thoracic artery.
d) Breast is attached to the pectoralis major.
e) Lymphedema causes Paeu'd orange appearance.

74) Venous blood from breast drains to,

a) Internal thoracic vein.
b) Intercostal veins.
c) Lateral thoracic vein.
d) Superior thoracic vein
e) Acromiothoracic vein

75) T/F regarding the mamillary gland.

a) It is a modified sebaceous gland.
b) It is in the deep fascia of the pectoral region.
c) Lactiferous sinuses are deep to the areolar tissue
d) Upper outer quadrant contains more glandular tissues
e) Nerve supply is provided by 2nd to 6th intercostal nerves.

76) True or false regarding breast,

a) It has an ectodermal origin
b) Extend from the lateral border of the sternum to the anterior axillary line
c) Lymphatics from the upper lateral quadrant drain to axillary lymph nodes
d) Montgomery gland is a modified sweat gland
e) Ligaments of cooper extend from subcutaneous tissues to the pectoral facia

Muscles of the upper limb

77) T/F regarding the muscles of the upper limb

a) Ability to elevate the arm above the head is impaired when the accessory nerve is damaged.
b) The subscapularis abducts the arm at the shoulder joint.
c) The anconeus abducts the ulna during pronation.
d) Pronation is stronger than supination.
e) Paralysis of the flexors of the wrist results in weakness of the grip.

78) T/F regarding the muscles of the upper limb

a) Ability to elevate the arm above the hand is impaired when the accessory nerve is damaged.
b) The subscapularis abducts the arm at the shoulder joint.
c) The anconeus abducts the ulna during pronation.
d) Pronation is stronger action than supination.
e) Paralysis of the flexors of the wrist results in weakness of the grip.

79) True or false

a) Serratus anterior arises from digitations of the lower 8 ribs.
b) Paralysis of serratus anterior results in winging scapula.
c) Serratus anterior is innervated by C_5 C_6 spinal nerves via long thoracic nerve.
d) Anterior muscles of the forearm can either flex the wrist or pronate the forearm.
e) The pronator muscles are innervated by a branch of the median nerve.

80) True or false

a) Deltoid is a medial rotator of the arm at the gleno-humeral joint.
b) Teres major muscle is supplied by upper subscapular nerve.
c) Dorsal scapular artery lies between trapezius and the rhomboid muscles.
d) Rhomboid major supplied by the dorsal scapular nerve.
e) The middle fibrous of trapezius protracts the scapula.

81) Medial rotators of the humerus

a) Pectoralis major.
b) Latissimus dorsi.
c) Subscapularis.
d) Teres major.
e) Infraspinatus.

82) What is the deep muscle which originate from radius and ulnar? (SBR)

a) Flexor digitorum profundus
b) Flexor indisus
c) Flexor pollicis longus
d) Abductor pollicis longus
e) Extensor carpi radialis longus

83) Muscles attached to coracoid process of scapula (SBR)

a) Brachialis and pectoralis minor.
b) Biceps and pectoralis minor.
c) Coracobrachialis and subclavius.
d) Serratus anterior and short head of biceps brachii.
e) Pectoralis major and minor.

84) The rotator cuff muscles

a) Comprise subscapularis, supraspinatus, infraspinatus and teres major.
b) Provide the greatest stabilizing forces at the shoulder joint.
c) Provide maximal support to the inferior aspect of the shoulder joint.
d) All are attached to the grater tuberosity of the humerus.
e) All are supplied by branches of the posterior cord of the brachial plexus.

85) The deltoid muscle

a) Is a bipinnate muscle.
b) Has anterior fibers that medially rotate the arm.
c) Has posterior fibers that extend the arm.
d) Has middle fibers that arise from the acromion.
e) Has middle fibers that abduct the upper limbs at the shoulder joint.

86) The deltoid muscle

a) Is a medial rotator of the arm at the glenohumeral joint.
b) Adducts the upper arm at the shoulder joint.
c) Is supplied solely by the axillary nerve.
d) Is multipinnate muscle.
e) Can flex the upper arm at the shoulder joint.

87) The deltoid muscle

a) Is a bipinnate muscle.
b) Rotates scapula laterally.
c) Is innervated by C_6 C_7 segments.
d) Is pierced by posterior branch of the axillary nerve.
e) Is not covered by the clavipectoral fascia.

88) Pectoralis major muscle

a) The clavicular part passes posterolaterally.
b) Is attached to the costal cartilage at 3rd, 4th, 5th ribs near their costochondral junctions.
c) Clavicular attachment is to the medial part of the inferior angle of the clavicle.
d) Receives nerve/nerves roots arise from lateral & posterior cord of the brachial plexus.
e) Is attached to the lateral lip of the lower part of the intertubercular groove.

89) The pectoralis major

a) Is attached to the floor of the intertubercular groove.

b) Is involved in flexion and lateral rotation of the arm at the shoulder joint.
c) Is used in climbing a rope by means of the upper limb.
d) Is inserted by a bilaminar tendon.
e) Extension of shoulder against resistance is by the sternocostal part.

90) The serratus anterior muscle

a) Gains attachment to all of the ribs.
b) Gains attachment to the medial border of the scapula.
c) Is attached by a group of accessory fibers to the medial bicipital groove.
d) Is an accessory muscle of respiration.
e) Is supplied by the thoracodorsal branch of the posterior cord of the brachial plexus

91) Teres major

a) Is attached to the dorsum of the scapula close to the inferior angle and to the medial lip of the intertubercular groove.
b) Is supplied by the lower subscapular nerve.
c) Help to extend the flexed upper arm.
d) The upper most and lateral parts of the posterior axillary fold is formed by it.
e) Is a lateral rotator of the arm.

92) Trapezius muscle

a) Dorsal scapular artery lies between trapezius and the rhomboid muscles.
b) Is the prime mover in the lateral rotation of the scapula.
c) Is supplied by a nerve arising from brachial plexus.
d) Lower fibers attached to the crest of the spine of scapula protract it which is opposed by serratus anterior.
e) Can be tested by making the patient raising his shoulder against resistance.

93) Which of the muscles and their attachments are correct,

a) Supraspinatus – Greater tubercle
b) Serratus anterior – Lateral border of scapula
c) Latissimus dorsi – Inferior angle of scapula
d) Long head of triceps – Supraglenoid tubercle
e) Coracobrachialis – Medial aspect of the coracoid process

94) Trapezius is attached to the

a) Occipital bone
b) Clavicle
c) Thoracic vertebra
d) Iliac crest
e) Acromion process

95) The latissimus dorsi muscle

a) Is attached to the lower six thoracic vertebrae.
b) Is attached to the lumbar and sacral vertebral spines.

c) Has no attachment to the chest wall.
d) Is attached to the inferior angle of the scapula.
e) Is a powerful flexor of the humerus at the shoulder joint.

96) Elevation of the scapula caused by

a) Upper fibers of trapezius
b) Levator scapulae
c) Rhomboid major
d) Supraspinatus
e) Latissimus dorsi

97) What is/are action(s) of rotator cuff?

a) Flexion
b) Abduction
c) Adduction
d) Lateral rotation
e) Medial rotation

98) Muscles arising from the interosseous membrane the forearm

a) Pronator quadratus
b) Flexor pollicis longus
c) Flexor digitorum profundus
d) Flexor digitorum superficialis
e) Extensor indicis

99) Following a month later, after a stab injury to his lateral chest wall, a coconut plucker developed a difficulty in climbing trees due to a weakness in upper limb. What is the most probable cause?

a) Paralysis of pectoralis major muscle.
b) Paralysis of serratus anterior muscle
c) Injury to suprascapular nerve
d) Damage to axillary nerve
e) Paralysis of latissimus dorsi muscle

100) T/F regarding the scapular muscles

a) Supraspinatus initiates the shoulder abduction
b) Infraspinatus is a medial rotator of shoulder
c) Rhomboids are lateral rotators of scapula
d) Teres major is the adductor of the shoulder joint
e) Paralyze of the subscapularis leads to wings scapula

101) Pectoralis minor muscle

a) Attached to 3rd 4th 5th costal cartilages
b) Supply by medial pectoral nerve
c) Protracts scapula
d) Is enclose by clavipectoral fascia
e) Lies anterior to 3rd part of axillary artery

102) Which of the pair is correctly given of following muscles and its action?

a) Serratus Anterior – Protraction
b) Lower Fibres of Trapezius – Retract
c) Upper Fibres of the Serratus Anterior - Lateral Rotation
d) Levator Scapulae - Medial Rotation
e) Upper Fibers of the Trapezius – Elevation

103) Supraspinatus muscle

a) It is supplied by dorsal scapular nerve
b) Abducts the shoulder
c) It is rotator cuff muscle
d) Is a lateral rotator of the shoulder
e) Tendon inflammation causes painful arch syndrome

104) Which of the following pairs correctly match with the movements of the shoulder /shoulder girdle and muscle that provide the movement,

a) Extension - Upper fibers of trapezius
b) Flexion – Latissimus dorsi
c) Extension - Teres major
d) Lateral rotation - subscapularis
e) Medial rotation - anterior fibers of the deltoid

105) T/F regarding rotator cuff muscles.

a) Teres major muscle is a rotator cuff muscle
b) They are attached to the bicipital groove
c) Subscapularis is attached to the lesser tubercle.
d) Teres minor is innervated by the axillary nerve.
e) Inflammation of supraspinatus tendon leads to painful arc syndrome.

106) Muscles originated from the lateral epicondyles of humerus?

a) Extensor carpi radialis brevis
b) Extensor digitorum
c) Flexor digitorum profundus
d) Flexor pollicis longus
e) Extensor digiti minimi

107) True or false regarding rotator cuff muscles,

a) Tendons merge to the joint capsule
b) Teres minor originates from the lower 1/3 of lateral border of scapula
c) Subscapularis inserts to the lesser tubercle
d) Infraspinatus laterally rotates the arm
e) Infraspinatus tendinitis leads to painful arch syndrome

Muscles of the flexor compartment

108) Coracobrachialis

a) Fused with long head of the biceps at its origin.
b) Pierce the musculocutaneous nerve.
c) It is a strong adductor of arm.
d) Separated from brachial artery by bicipital aponeurosis.
e) Resist dislocation of the shoulder joint.

109) The biceps muscle

a) Is attached to the scapula.
b) Has an intra-articular tendon.
c) Is attached to the humerus.
d) Has an aponeurosis passing to the dorsal surface of the radius.
e) Is a powerful pronator of the forearm

110) The brachioradialis muscle is

a) Attached proximally to the lower third of the lateral supracondylar ridge of the humerus.
b) Attached distally to the dorsal aspect of the base of the 3rd metacarpal bone.
c) A flexor of the elbow.
d) A pronator of the forearm.
e) Supplied by the posterior interosseous nerve.

111) The flexor digitorum superficialis muscle,

a) Flex the middle phalanx.
b) Extend metacarpophalangeal joint.
c) Has an attachment to humerus.
d) Is innervated by anterior interosseous branch of median nerve.
e) Gives an attachment to lumbricals.

112) Flexor digitorum superficialis

a) Origin from the coronoid process of the ulna.
b) Share with flexor digitorum profundus in a same synovial sheath.
c) Four tendons enter the palm through the carpal tunnel.
d) Flexors all interphalangeal joints.
e) Is supplied by the median nerve in the cubital fossa.

113) Regarding Flexor digitorum superficialis muscle

a) Has a humeral head.
b) Insert to distal phalanx of middle four fingers.
c) Flex all interphalangeal joints.
d) Lies superficial to the median nerve.
e) Is supplied by ulnar nerve.

114) Regarding the long flexors

a) The tendon of flexor digitorum superficialis divides over the metacarpal bases and is attached to the bases of the middle phalanges of the fingers.
b) The 1st dorsal interosseous arises from the adjacent sides of the profundus tendons to index and middle fingers.
c) A deep cut two inches above the wrist will divide four tendons of deep flexors.
d) Flexor tendons of a finger belong to separate muscle having same motor supply.
e) Are held by lumbricals during the flexion of interphalangeal joints.

115) T/F regarding the forearm muscles

a) All flexor muscles are supplied by median nerve.
b) 2nd and 5th tendons of flexor digitorum superficialis go over 3rd and 4th tendons.
c) Flexor digitorum superficialis causes only for flexion of IP joint.
d) Pronator quadratus initiate the pronation.
e) Tennis elbow is due to repeated forceful movements of muscles arising from common flexor origin.

116) Muscles origin from medial epicondyle of humerus include

a) Flexor carpi ulnaris
b) Flexor digitorum superficialis
c) Flexor digitorum profundus
d) Flexor policis longus
e) proneter teres

117) Flexor pollicis longus

a) Supplied by anterior interosseous nerve.
b) In paralysis flexion of the MP joint is lost.
c) In paralysis flexion of the interphalangeal joint is lost.
d) Muscle is paralyzed in carpal tunnel syndrome

118) Flexor digitorum profundus muscle

a) Arises from both ulna and radius
b) Insert on to the distal phalanges of medial four fingers
c) Lies superficial to the ulnar nerve
d) Supplied by C8 &T1 roots
e) Paralysis causes ulnar claw hand

119) T/F regarding the biceps brachi

a) Its long head is within the shoulder joint.
b) It is a flexor of shoulder joint.
c) It is supplied by ulnar nerve.
d) Attached to the coronoid process.
e) Can be tested by flexion against resistance.

120) T/F regarding the flexor compartment of the forearm,

a) Ulnar artery passes deep to the deep head of the pronator teres muscle.
b) Flexor digitorum superficialis muscle is supplied by the anterior interosseous nerve.
c) Radial pulse can be felt just medial to the tendon of flexor carpi radialis.
d) Interosseous recurrent artery arises from the common interosseous artery.
e) Ulnar claw is more obvious when the ulnar nerve is divided in the elbow than in the wrist.

121) T/F regarding flexor digitorum profundus,

a) It is attached to both radius and ulna bones.
b) Is attached to the distal phalanx of the thumb.
c) It passes superficially to the flexor retinaculum.
d) it is innervated by ulnar nerve
e) It is tested by flexing interphalangeal joints.

122) True or false regarding flexor compartment of the arm?

a) Flexor carpi radialis muscle lies in its superficial muscle layer
b) All the muscles in this compartment arise from the medial epicondyle
c) The whole flexor compartment is supplied by the median nerve
d) At the wrist palmaris longus muscle is lateral to flexor carpi radialis tendon
e) Flexor policis longus arises from ventral surface of middle two quarter of ulnar

Muscles of the extensor compartment

123) T/F about extensor muscles of the upper limb

a) Inflammations of common extensor origin lead to golfer's elbow.
b) Extensor digitorum and extensor digiti minimi runs in same compartment in extensor retinaculum.
c) Extensor pollicis brevis attached to lateral supracondylar line.
d) Interossei together with abductor digiti minimi causes abduction and adduction of the finger.
e) Adductor pollicis inserts into trapezium.

124) Regarding extensor muscles of the forearm

a) Damage to the deep branch of radial nerve in supinator will spare the function of extensor carpi radialis brevis.
b) Supinator is powerful lateral rotator of radius.
c) Flexor digitorum profundus covers all interphalangeal joints.
d) Abductor pollicis longus extends the thumb at the carpometacarpal joint.

125) The triceps muscle

a) Is attached to the infraglenoid tubercle of the scapula.
b) Is attached to the borders of the radial groove of the humerus.
c) Is attached to the ulnar olecranon.

d) Acts mainly at the shoulder joint.
e) Is supplied by the median nerve.

126) The abductor pollicis longus muscle

a) Is attached to the interosseous membrane.
b) Tendon passes deep to both extensor carpi radialis longus and brevis.
c) Is attached to the base of the proximal phalanx of the thumb.
d) Produces extension at the thumb's carpometacarpal joint.
e) Possesses a separate synovial sheath around its tendon.

127) The extensor digitorum muscle

a) Is attached proximally to the anterior aspect of the lateral epicondyle of the humerus.
b) Covers the proximal phalanges by dorsal expansions of its four tendons.
c) Is attached to the bases of the proximal phalanges of the four fingers.
d) Has small tendinous slips to the dorsal expansion from the lumbrical and interosseous muscles.
e) Is supplied by the radial nerve.

128) The supinator muscle

a) Forms part of the floor of the cubital fossa.
b) Is attached to the medial epicondyle of the humerus.
c) Is attached to the proximal end of the ulna.
d) Is attached to the proximal end of the radius.
e) Is supplied by the ulnar nerve.

129) Extensor pollicis longus

a) Lies medial to the dorsal tubercle of the radius.
b) Crosses the tendons of extensor carpi radialis longus and brevis.
c) Can abduct the extended thumb.
d) Has its origin from the middle 1/3 of the posterior surface of the ulna.
e) Is attached to base of the 1st metacarpal bone.

130) True or false about the posterior compartment of forearm?

a) Brachioradialis is a content.
b) Extensor pollicis longus originates from lateral epicondyle of the humerus.
c) Anconeus supinates the forearm.
d) Cutaneous supply is by deep branch of radial nerve.
e) All muscles are supplied by the posterior interosseous nerve.

131) Extensor indicis muscle

a) Originates from the posterior surface of the shaft of the radius
b) Inserted directly to the base of the middle phalanges of the index
c) Extends metacarpophalangeal joints of the index finger
d) Tendon is in common synovial sheath with extensor digitorum

e) Damage results loss of extension of interphalangeal joint

132) Which of the following muscles originate from interosseous membrane?

a) Extensor indicis
b) Extensor pollicis longus
c) Extensor pollicis brevis
d) Abductor pollicis longus
e) Supinator

133) The supinator muscle

a) Originated from annular ligament
b) Laterally related to neck of radius
c) Supplied by muscular cutaneous nerve
d) Is the prime mover in supination
e) Make the floor of the cubital fossa

134) A young man had a dorsal cut injury over radial styloid process. During surgery extensor pollicis longus muscle and the muscle immediately lateral to it was damaged. What was the other damaged muscle? ((SBR)

a) Abductor pollicis longus
b) Extensor carpi radialis brevis
c) Extensor carpi radialis longus
d) Extensor digitorum
e) Extensor pollicis brevis

135) Contents of Posterior compartment of Forearm,

a) Extensor carpi radialis Longus
b) Extensor digitorum
c) Abductor pollicis Longus
d) Superficial branch of radial nerve
e) Radial artery

Muscles of the hand

136) Regarding muscles of the hand

a) Lumbricals arise from the flexor digitorum profundus tendons.
b) Lumbricals flex the MP joint.
c) All interossei muscles arise from two heads from adjacent metacarpal bone.
d) Dorsal interossei abducts the fingers.
e) Palmar interossei abduct the fingers.

137) Regarding muscles of the hand

a) If there is a lesion of musculocutaneous nerve sensation may lost on the skin over the thenar eminence.
b) Opponence pollicis is attached to the anterior surface of the middle phalanx.
c) Skin over the dorsal surface of the terminal phalanx of the index finger is supplied by the branches of radial nerve.
d) All the interossei muscles are supplied by ulnar nerve.
e) Medial two lumbricals are bipinnate.

138) The1st lumbrical muscle

a) Originate from flexor digitorum profundus tendon to lateral side of the index finger.
b) Attached to the extensor expansion.
c) First lumbrical attached to the thumb.
d) Flexes interphalangeal joint of the index finger.
e) Is supplied by recurrent branch of the median nerve.

139) About lumbricals

a) Attach to the medial side of the extensor expansion.
b) Pass anterior to the deep transverse metacarpal ligaments.
c) Receive their segmental innervation from T_1.
d) First lumbrical is supplied by ulnar nerve.
e) If paralyzed extension of IP joint is impaired.

140) T/F regarding lumbricals

a) Lumbricals extend the fingers at IP joints and flex at MP joints.
b) First two lumbricals are supplied by the median nerve.
c) Lumbricals are attached to the flexor digitorum superficialis.
d) The 1st lumbrical is unipinnate.
e) Lumbricals are inserted into extensor expansion.

141) True/False of lumbricals

a) They are originated from flexor digitorum superficialis tendon
b) Second lumbrical is innervated by the median nerve
c) They flex the interphalangeal joints
d) They are inserted into the base of the proximal phalanges
e) Third lumbrical is bipinnate

142) Lumbricals

a) Flex metacarpopharyngeal joints
b) Extend inter pharyngeal joints
c) Lumbricals adduct the fingers at metacarpopharyngeal joints
d) Lumbricals arise from the tendons of flexor digitorum profundus
e) Their action can be assessed by asking the patient to hold a piece of paper between two fingers

143) True/False of the hand?

a) Dorsal interossei abduct hand.
b) Lumbricals flex metacarpophalangeal joints.
c) All lumbricals are supplied by median nerve.
d) Small muscles of the hand are supplied by C8.
e) Flexor pollicis brevis flexes the metacarpophalangeal joint of the thumb.

144) Palmar interossei

a) Origin from body of the metacarpals.
b) Are unipinnate muscles.
c) Are innervated by deep branch of ulnar nerve.
d) Extension of the interphalangeal joints.
e) Can be tested by the abduction the fingers against the force.

145) Lumbrical muscles,

a) are attached to FDS tendon.
b) extend the interphalangeal joint of the thumb.
c) flex the metacarpophalangeal joint of the 5th finger.
d) Adduct the fingers towards the midline of the hand
e) when paralyzed, claw hand occurs.

146) True or false regarding the small muscles of the hand,

a) Adductor pollicis is supplied by the Median nerve
b) The medial two lumbricals are supplied by the ulnar nerve
c) The 1st dorsal interosseous flexes the metacarpophalangeal joint of thumb
d) Lumbricals are located deep to the mid palmar space
e) They are innervated by T2 segment

Muscle testing

147) True or False

a) Trapezius is tested by shrugging the shoulder against the resistant.
b) Abductor pollicis muscle can be tested with pen touch test.
c) Flexor digitorum profundus and flexor digitorum superficialis cannot be tested separately.
d) Bicep muscle tested by flexing the pronated forearm against resistance.
e) Brachioradialis is tested by flexing the mid pronated forearm against the resistance.

148) T/F regarding the testing of the muscles

a) Foment test is used to check the integrity of superficial branch of ulnar nerve.
b) Deltoid can be tested by shrugging test.
c) By clasping can check the integrity of the median nerve.
d) Bicep muscle is tested by asking the flexed pronated forearm against a resistance.

149) T/F regarding testing of muscles

a) Trapezius muscle can be tested by asking the patient to shrug the shoulder.
b) Brachioradialis muscle is tested by flexing forearm against resistance while the forearm is in mid prone position.
c) In the hand damage of flexor carpi ulnaris attempted to flex the hand causes abduction of the head.
d) Interossei muscles are tested by keeping the head flat on the table.
e) The integrity of the ulnar nerve can be checked by performing the pen touching test.

150) Inability to hold a piece of paper between 2nd & 3rd fingers,

a) Paralysis of Palmar interossei
b) Paralysis of Dorsal interossei.
c) Paralysis of Dorsal interossei.
d) Damage to superficial branch of Ulnar Nerve
e) Damage to deep branch of Ulnar nerve

Brachial plexus

151) T/F regarding brachial plexus

a) Formed by dorsal rami of C_5 to T_1 roots.
b) Divisions are posterior to the medial 1/3 of clavicle.
c) Supraclavicular nerves arise from the middle trunk.
d) Anterior divisions of all 3 trunks unite to form the lateral cord.
e) Cords lie lateral to the 1st part of axillary artery.

152) True or false regarding brachial plexus

a) A nerve derived from the lateral cord provides motor innervation to the flexor muscles of the area
b) The cords at the brachial plexus lie posterior to the 1st part of the axillary artery
c) Nerve of the lateral cord do not innervate skin not the arm
d) Abduction of the shoulder joint disturbed on Erb's palsy
e) All the muscles supplied by ulnar nerve are paralyzed in Klumpke's palsy

153) Regarding the brachial plexus

a) Formed mainly by 5 anterior rami in the neck and in the axilla.
b) Innervates the free upper limb but not the shoulder musculature.
c) The roots are behind the clavicle.
d) The 1st thoracic nerve crosses the 1st rib.
e) The sensory fibers pass in both roots of the median nerve.

154) The posterior cord of brachial plexus

a) Contains only the fibers from the 7th and 8th cervical spinal nerves.
b) Gives off a branch which joins the median nerve.
c) Is lateral to the axillary artery in the glenohumeral joint.
d) Gives off branches which supply the subscapularis, teres major and latissimus dorsi.
e) Contains fibers which supply the muscles of the back of the forearm.

155) Regarding the upper trunk of the brachial plexus

a) Contains fibers which supply the skin on the medial aspect of the arm.
b) Supplies muscles which produce abduction, flexion and medial rotation at the shoulder joint.
c) If damaged, results in loss of adduction at the shoulder joint.
d) If damaged, the functions of the interossei are lost.
e) If damaged, there is loss of sensation along the medial side of the forearm.

156) Regarding the brachial plexus

a) Nerve to subclavius lies posterior to the brachial plexus.
b) Lateral pectoral nerve and long thoracic nerve have the same root value.
c) Brachial plexus is covered by the axillary sheath.
d) Cords are lying medial to the 2nd part of the axillary artery.
e) Brachial plexus ends at the distal border of the pectoralis minor.

157) T/F regarding the brachial plexus

a) Musculocutaneous nerve contains C_7, C_8, T_1 segments.
b) Divisions lie behind the subclavius.
c) Lesion of the lower trunk results in sensory impairment at the tip of the middle finger.
d) Upper root lesion result wrist drop.
e) Ulnar nerve descends through the posterior compartment of the arm throughout its course.

158) T/F regarding the brachial plexus

a) Trunks lie behind the middle 3rd of the clavicle.
b) Stab injury at the posterior triangle causes an inability to initiate the abduction of the shoulder joint.
c) The supraclavicular nerve is the only nerve to arise from the trunks.
d) Damaged to thoracodorsal nerve causes winged scapula.
e) Brachial plexus emerge cut between anterior and posterior scalene muscles.

159) T/F regarding the brachial plexus

a) The trunks lie behind the middle 1/3 of the clavicle.
b) Dorsal scapular nerve arises from the upper trunk.
c) Supination impaired in Erb's palsy.
d) Lower trunk lesion leads to complete claw hand.
e) The lateral cord continuous as the ulnar nerve.

160) Regarding the brachial plexus

a) Sympathetic preganglionic fibers are carried with from T1 root.
b) Trunks of brachial plexus lie in the middle 1/3 of clavicle.
c) Lateral pectoral nerve lies medial to middle pectoral nerve at the root of the neck.
d) Dorsal scapula nerve arises from upper trunk.
e) Lower trunk lesions lead to paralysis of serratus anterior.

161) Regarding the brachial plexus (SBR)

a) It has nerve roots of C_5, C_6, C_7 & T_1.
b) In the axilla medial cord lies medial to axillary artery.
c) Radial nerve injury causes wrist drop.
d) Thoracodorsal nerve contains $C_6 – C_8$ nerve roots.
e) Klumpke's palsy is due T_1 nerve injury.

162) The brachial plexus

a) Is usually formed by the ventral rami of the lower four cervical and first thoracic nerves.
b) Has its roots situated posterior to the scalenus anterior muscle.
c) Contains three trunks which lie in the neck.
d) Contains three cords which lie in the neck.
e) Has a posterior cord which receives contributions from all five roots of the plexus.

163) Damaged to C_5-C_6 roots

a) Sensation loss in the lateral surface in the forearm.
b) Weakness in supination of the forearm.
c) Cannot extend the elbow.
d) The hand being in the porter's tip position.
e) Cannot abduct the arm.

164) Injury to lateral cord of brachial plexus results in paralysis of (SBR)

a) Biceps brachii
b) Brachialis
c) Supraspinatus
d) Pectoralis major
e) All muscle that is supplied by median nerve.

165) T/F regarding brachial plexus

a) Its divisions lie behind the middle third of the clavicle
b) Lower trunk carries C8 and T1 fibers
c) Median nerve is formed by lateral cord
d) Upper trunk lesion cause Erb's palsy
e) Long thoracic nerve formed by posterior cord

166) Damage to the lateral cord of brachial plexus results in

a) Weakness of pronation.
b) Impairment of opposition of thumb.
c) Derangement of flexion at the MP joint of the ring finger.
d) Loss of sensation over the anatomical snuff box.
e) Impairment of the flexion at the wrist joint.

167) T/F regarding brachial plexus,

a) Dorsal rami of the C5 to T1 contribute in formation.
b) Divisions lie in the axilla.
c) Root values of the axillary nerve are C5-C6
d) Suprascapular nerve arises from the upper trunk.
e) Ulna nerve arises from the lateral cord.

Nerves of the Upper limb

168) Regarding nerves of upper limb

a) Cords of brachial plexus can be palpated on the head of humerus.
b) Axillary nerve can be damaged in surgical neck of humerus.
c) Supraclavicular nerves can be palpated over clavicle.
d) Ulnar pulse is felt just medial to ulnar nerve.
e) Damage to upper trunk of brachial plexus cause loss of flexion at elbow.

169) Regarding the nerves of the upper limb

a) The first nerve to leave the neurovascular bundle is the radial nerve.
b) Ulna nerve passes through the carpel tunnel with the ulna artery.
c) All the short muscles of the hand are innervated by ulna nerve.
d) Superficial branch of the radial nerve becomes the lateral cutaneous nerve of forearm.
e) Median nerve passes behind the deep head of pronator quadrates.

170) Opposition of the thumb is dependent upon the functional integrity of

a) Anterior interosseous nerve and ulnar nerve.
b) Median nerve and ulnar nerve.
c) Musculocutaneous nerve and median nerve.
d) Radial nerve and median nerve.
e) Ulnar nerve and radial nerve.

171) Regarding the nerves of the upper limb

a) Supraclavicular nerves are palpable.
b) Ulnar nerve at the wrist lies lateral to the ulnar pulse.
c) Posterior interosseous branch of radial nerve is located on just distal to the head of the radius.
d) Median nerve crosses the brachial artery lateral to medial at the arm.
e) Median nerve lies lateral to the flexor digitorum superficialis tendon at the wrist.

172) Regarding the nerves of the upper limb

a) Anesthesia over the lower part of the skin over the deltoid is a feature of axillary nerve damage.
b) Wrist drop is present when the posterior interosseous nerve is damaged.
c) Erb's paralysis is associated with Horner syndrome.
d) Ulnar nerve damaged at wrist is difficult to diagnose clinically.
e) Median nerve damage at wrist causes "ape" like thumb.

173) True or false

a) The skin of the middle of the middle finger is supplied by median and radial nerve.
b) A patient with wasting of thenar muscles but normal flexion and extension of arm and forearm has a lesion of median nerve in the cubital fossa.
c) The skin over the thenar eminence is supplied by ulnar nerve.
d) The skin of the back of the chest is innervated by anterior and posterior rami of thoracic spinal nerves.
e) Damaged to the middle cervical sympathetic ganglion render the skin over the dorsum of the hand hot and dry.

174) True or False

a) 7th cervical segment supplies the skin on the dorsal and palmar surfaces of the middle finger.
b) If the ulnar nerve is damaged at the elbow, all the muscles which are supplied by C_8 and T_1 are paralyzed.
c) In the lesion of the 2nd thoracic nerve the sensory loss is well defined at the back of the arm.
d) The median nerve lies between the biceps and brachialis muscle.
e) When the radial nerve damaged in the radial groove, the patient does not exhibit any deformity.

175) Suprascapular nerve

a) Supplies levator scapulae.
b) Contains fibers from the dorsal ramus of C_5 & C_6.
c) Pierces coracobrachialis.
d) Arises from the lateral cord.
e) Supplies the teres minor.

176) Which statement is correct regarding the nerves of the forearm

a) Extensor pollicis longus run around the medial side of the dorsal tubercle of radius.
b) Radial nerve arises sensation from the thumb.
c) Musculocutaneous nerve is sensory nerve.
d) Ulnar nerve is composed of C_8 and T_1 segments.
e) Damage to the ulnar nerve causes to claw hand.

177) True / False

a) Radial nerve passes to posterior compartment through quadrangular space.
b) Medial intermuscular septum is pierced by ulnar nerve.
c) Upper lateral cutaneous nerve is supplied to skin over the upper part of deltoid.
d) Segmental innervation of lateral part of arm is by C6.
e) Median nerve is liable to injuries of supracondylar fractures.

178) T/F about the nerves of the upper arm?

a) Ulnar nerve injury in wrist more prominent than at the elbow.
b) Hand functions are more affected by ulnar nerve than median nerve.
c) Radial nerve injury in spiral groove of arm cause wrist drop.
d) Upper trunk lesions of brachial plexus results in weak shoulder abduction.
e) Injury to long thoracic nerve results in paralysis of serratus anterior.

179) A woman undergoes a mastectomy, later found with impaired sensation in the upper medial side of the arm and axilla. What is the nerve supply to the medial side of the upper part of the arm and axilla? (SBR)

a) Medial cutaneous nerve of the arm
b) Intercostobrachial nerve
c) Long thoracic nerve
d) Axillary nerve
e) Thoracodorsal nerve

180) A person fell down from a tree. His arm was hyperabducted. He was unable to keep a piece of paper between the ring and middle finger. There was a loss of sensation in the middle side of the forearm. Damaged nerve is? (SBR)

a) Axillary nerve
b) Upper subscapular nerve
c) T1 root
d) Musculocutaneous nerve
e) C8 root

181) WOF muscles are correctly matched with their nerve supply?

a) Supraspinatus – Subscapular nerve
b) Trapezius – Accessory nerve
c) Pectoralis minor – Lateral pectoral nerve
d) Deltoid – Axillary nerve
e) Rhomboid – Dorsal scapular nerve

182) Which of the following pairs correctly matching regarding the segmental nerve supply and area of skin supplied by it?

a) C4 – Tip of the shoulder
b) C6 – Thumb
c) C7 – Little finger
d) C8 – Elbow
e) T1 – Little finger

183) 40-year-old male features suggestive of C5/C6 nerve roots injured following trauma?

a) Loss of sensation over floor of axilla.
b) Loss of sensation over the insertion of deltoid muscle.
c) Loss of shoulder extension.
d) Weakness in the elbow extension.
f) Weakness in the flexion of ring finger.

184) A 32-year-old man present to emergency department following a crush injury to right elbow region. On examination, adduction of right thumb is possible. The nerve conduction has confirmed a single nerve injury. What is the abnormality observed?

a) Impair thumb abduction.
b) Los of sensation over medial side of dorsal side of right hand.
c) Weakness in the extension of interphalangeal joint of middle finger.
d) Patch of anesthesia in the posterior aspect of the forearm.
e) Wrist drop

185) 40 years old women presents with loss of sensations in the floor of her right axilla and upper medial 1/2 of the right arm following mastectomy and removal of axillary lymph nodes for a malignance lesion of her right heart. above features are suggested of a nerve injury during the procedure. What's the nerve injury?

a) intercostobrachial nerve
b) axillary nerve
c) long thoracic nerve
d) medial cutaneous nerve of the arm
e) Thoracodorsal nerve

186) Which statement is correct regarding the nerves of the forearm,

a) Extensor pollicis longus run around the medial side of the dorsal tubercle of radius
b) Radial nerve arises sensation from the thumb
c) Musculocutaneous nerve is sensory nerve
d) Ulnar nerve is composed of C8 and T1 segments
e) Damage to the ulnar nerve causes to claw hand

Axillary Nerve

187) Axillary nerve

a) Roots $C_5 – C_7$.
b) Supplies to teres major.
c) Is a pure motor nerve.
d) Damage during humeral shaft fractures.
e) Damage leads to weakness of deltoid muscle.

188) The axillary nerve

a) Has no cutaneous branches.
b) Passes inferior to teres major and supplies it.
c) Contains fibers from C5 and C6 spinal nerves.
d) Is medial to the surgical neck of humerus.
e) Supplies subscapularis, deltoid and teres minor muscles.

189) Damage to the axillary nerve may affect

a) Abduction of the arm.
b) Extension off the arm.
c) Medial rotation.
d) Lateral rotation.
e) Adduction of the arm.

190) The axillary nerve

a) Damage results in severe weakening of abduction.
b) May be damaged in downward dislocation of the head of the humerus from the shoulder joint.
c) Accompanies the anterior circumflex humeral artery with its anterior branch.
d) Anterior branch of axillary nerve runs posterior to the surgical neck.
e) If damaged in axilla results in impairment of deltoid and sensory loss over the lower half of deltoid muscle.

Musculocutaneous nerve

191) The musculocutaneous nerve

a) Originates from the medial cord of the brachial plexus.
b) Pierce the deep fascia just above the elbow.
c) Innervates pronator teres.
d) Supplies the skin of lateral aspect of the forearm.
e) Innervates the elbow joint.

192) The musculocutaneous nerve

a) Is a terminal branch of the posterior cord of the brachial plexus.
b) Descends in the arm between biceps and brachialis.
c) Supplies coracobrachialis.

d) Supplies cutaneous branch to the radial side of the forearm.
e) Ends up as the medial cutaneous nerve of the forearm.

Median nerve

193) T/F regarding median nerve

a) Arises by two roots, one from the medial cord and the other from lateral cord of the brachial plexus.
b) Does not contain fibers originating the 1st thoracic nerve.
c) Is commonly the motor nerve to the abductor pollicis brevis muscle.
d) Lies deep to pronator teres and flexor digitorum superficialis.
e) Passes through the carpal tunnel lying medial to flexor carpi radialis.

194) The median nerve

a) Lies on the superficial surface of the flexor digitorum superficialis.
b) If sectioned at the elbow, results in wasting of the thenar muscles.
c) Supplies the lateral two palmar interosseous muscles
d) At the wrist lies medial to the tendons of flexor digitorum superficialis.
e) Is lateral to the tendon of biceps in cubital fossa.

195) The median nerve

a) Damaged at the wrist causes inability to oppose the thumb.
b) At mid arm level, it crosses brachial artery anteriorly.
c) It travels within the neurovascular compartment of the arm.
d) In carpal tunnel syndrome numbness occurs over the thenar eminence.
e) Supply all the interossei of the hand.

196) Median nerve supply for (SBR)

a) Opponens pollicis.
b) Supinator.
c) Flexor carpi ulnaris.
d) Brachioradialis.
e) Flexor digitorum profundus.

197) Damage to the median nerve in the arm would affect the

a) Supination of the forearm.
b) Pronation of the forearm.
c) Extension of the wrist.
d) Opposition of the thumb.
e) Flexion of thumb.

198) If the median nerve is damaged in the arm

a) Forearm is in prone position.
b) Sensory loss over the dorsum of the distal phalanges of lateral 3 fingers occurs.
c) There is loss of flexion in the terminal phalanx of the thumb.
d) There is wide spread sensory loss over the back of the forearm.
e) The flexed forearm cannot be actively extended at the elbow.

199) If the median nerve cut down at the wrist

a) Loss of sensation over whole of the back of the index finger.
b) Adduction of the thumb is lost.
c) Tip of the thumb can't be brought into contact with the tip of the little finger.
d) Inability to flex the index finger at metacarpophalangeal joint.
e) Inability to flex the index finger at the interphalangeal joint.

200) Regarding the median nerve damage at wrist

a) Sensory loss occurs at dorsal side of thumb.
b) Cannot abduct the thumb.
c) Cannot contact with little finger by using tip of the thumb.
d) Flexion of metacarpophalangeal joint of thumb is impossible.
e) Cannot extend index finger at interphalangeal joints.

201) Damage to anterior interosseous nerve

a) Loss of flexion at metacarpophalangeal joint of thumb.
b) Loss of flexion at interphalangeal joint of thumb.
c) Impaired sensation over the lateral 2/3 of the arm.
d) Loss of flexion of distal interphalangeal joint of ring finger.
e) Loss of the flexion of distal interphalangeal joint of little finger.

202) The anterior interosseous nerve

a) Arises from the median nerve in the cubital fossa.
b) Innervates the flexor digitorum superficialis.
c) Gives articular branches to the elbow joint and wrist joint.
d) Descends in front of the interosseous membrane.
e) Is accompanied by anterior interosseous artery.

203) T/F regarding the muscles which are supplied by the median nerve?

a) Extensor carpi radialis brevis
b) Abductor policis longus
c) Flexor policis longus
d) Flexor digitorum superficialis
e) Flexor carpi radialis

Ulnar nerve

204) The ulnar nerve

a) Is the first structure to leave the main neurovascular bundle of the arm.
b) Pierce the medial intermuscular septum of the arm.
c) Supply the adductor pollicis.
d) Provides cutaneous supply to the anterolateral surface of the forearm.
e) Damage at the wrist, produces less deformity than the damage at the elbow.

205) Regarding the ulnar nerve

a) Dorsal branch is given off in the distal 1/3 of the forearm.
b) Supplies the medial half of the flexor digitorum profundus.
c) Gives the medial cutaneous nerve in the axilla.
d) The deep branch supplies the 1st dorsal interosseous muscle.

e) If cut at the elbow, flexion at the MP joints of the ring and little finger is lost, if their IP joints are kept extended.

206) The ulnar nerve

a) Is posterior to the medial epicondyle of the humerus.
b) If damaged at the elbow, the movement of the opposition of the thumb is usually lost.
c) Supplies abductor pollicis brevis muscle.
d) Lies medial to the hook of the hamate and lateral to the pisiform.
e) Supplies all interosseous muscles.

207) Regarding the ulnar nerve

a) It is deep to the flexor retinaculum.
b) Contains fibers from only the 1st thoracic spinal nerve.
c) It lies on the ulnar collateral ligament of the elbow joint, as it enters forearm.
d) Deep branch passes between the hypothenar muscles and the long flexor tendons to the little finger.
e) Claw hand is more marked when damaged at the elbow than when damaged at the wrist.

208) In damage to the ulnar nerve above elbow

a) Thumb cannot be abducted or adducted.
b) Hand can be adducted without any difficulty.
c) There is loss of sensation of the medial surface of the lower part of the forearm.
d) Part of flexor digitorum superficialis is paralyzed.
e) There is loss of sensation in the back of the index finger.

209) The ulnar nerve

a) Divides into superficial and deep branches in the forearm.
b) Supplies a part of flexor digitorum profundus.
c) Lateral to the pisiform bone.
d) Gives a cutaneous branch to the dorsum of the hand.
e) If damaged at the elbow results in inability to abduct the fingers.

210) In damaged to ulnar nerve at the elbow joint

a) Loss of sensation over the dorsal surface of middle phalanx of the 5th metacarpal.
b) Inability to extend at the interphalangeal joint of the ring finger.
c) Thumb movements are not affected.
d) Wrist drop.
e) Weakening of the wrist flexion.

211) Ulnar nerve injury at the wrist causes

a) Sensory loss of the dorsal surface medial 1/3 of hand

b) Impairment in adduction of the thumb
c) Prominent claw hand compared to the ulnar nerve injury at elbow
d) Wasting of thenar muscles
e) More prominent functional impairment than median nerve injury

212) Injury to ulnar nerve at elbow results in,

a) lateral 2 lumbricals are paralyzed
b) Positive froment's test
c) Ulnar paradox occurs
d) Results in ape hand
e) Interossei muscles are paralyzed

Radial nerve

213) Regarding the radial nerve

a) Direct continuation of posterior cord of the brachial plexus.
b) Have fibers from T_1.
c) Supplies the lateral 3 ½ of palm.
d) Mainly supplies extensor muscles of the forearm.
e) If damaged at elbow claw hand is a feature.

214) The radial nerve

a) Lies in the groove of humerus between origins of medial and lateral heads of triceps.
b) When damaged at the elbow it is unable to comb the hair.
c) Supplies all extensors of the forearm.
d) Superficial branch is both motor and sensory.
e) Divides to deep and superficial branches before piercing the lateral intermuscular septum.

215) The radial nerve

a) Lies 1st behind the axillary artery.
b) At the upper third of the humerus radial nerve pierce lateral intermuscular septum.
c) Becomes the lateral cutaneous nerve of the forearm.
d) Damage to the posterior interosseous nerve causes wrist drop.
e) Radial nerve injury in the axilla causes loss of supination.

216) Regarding the radial nerve

a) It is only nerve supply triceps muscle.
b) Accompanies the radial artery in part of its course.
c) It is entirely cutaneous in its distribution distal to the elbow joint.
d) Lies deep to the deep fascia from the elbow to the wrist.
e) Supplies only the distal phalanges of the lateral 4 digits in the hand.

217) Regarding the radial nerve

a) It supplies to dorsal interosseous muscles.
b) The deep branch pierces the anconeus muscle.
c) Flexor pollicis longus is supplied by posterior interosseous nerve.
d) The nerve is damaged in supracondylar fracture of the humerus.
e) It is the largest nerve in the lateral cord of the brachial plexus.

218) Radial nerve

a) If paralyzed leads to Erb's palsy.
b) Lies between medial and lateral head of triceps.
c) Supplies cutaneous branches to radial side of the forearm.
d) Gives off the posterior interosseous nerve at the level of the lateral epicondyle.
e) It is the terminal branch of the posterior cord.

219) If the radial nerve completely sectioned in the axilla

a) The patient unable to have a firm grip with that hand.
b) There is loss of supination in the flexed forearm.
c) There is widespread sensory loss over the back of the forearm.
d) The fingers cannot be extended at the MP joints.
e) The flexed forearm cannot be actively extended at the elbow.

220) The damage of the radial nerve in the spiral groove

a) Wrist drop
b) Loss of extension
c) Loss of supination
d) Loss of pronation
e) Loss of abduction of the thumb

221) Deep branch of the radial nerve

a) Arises from the neck of the radius

b) Pierce supinator muscle
c) Laterally related to posterior interosseous artery
d) Is anteriorly related to the extensor pollicis longus tendon
e) Supplies an articular branch to the elbow joint

222) 34 years old person present with a radial nerve injury. Following the accident at distal end humerus, at the level of elbow. What is the most likely paralyzed muscle? (SBR)

a) Triceps Brachi
b) Brachioradialis
c) Anconeous
d) Brachialis
e) Brachialis

223) The deep branch of radial nerve

a) Divide in front of the medial epicondyle of humerus
b) Pierce the supinator muscle
c) Related to the neck of humerus
d) Supplies abductor pollicis longus
e) Supplies carpal bones

224) Radial nerve damage above the supracondylar ridge of the humerus results,

a) Wrist drop
b) Loss of extension of the elbow
c) Elbow flexion
d) Sensory loss over lateral 1/3rd of dorsal aspects
e) Supination

Upper limb vessels & nerves

225) Regarding upper limb vessels

a) Radial artery runs deep to extensor pollicis brevis.
b) Superficial palmar arch is distal to deep palmar arch.
c) Basilic vein runs in the posteromedial aspect of the forearm.
d) Brachial artery is protected by the underlying bicipital aponeurosis.
e) Cephalic vein begins anterior to the radial styloid process.

226) T/F regarding neurovascular bundle of the arm

a) Contain radial nerve in the middle third of the arm.
b) Has the medial cutaneous nerve of the arm as a component.
c) Median nerve leaves the neurovascular bundle at the middle of the arm.
d) Contains venae commitantes of the brachial artery in the middle of the arm.

e) Contains ulnar nerve above the insertion of coracobrachialis.

227) T/F regarding the blood vessels of the upper limb

a) Brachial artery bifurcates into ulnar and radial arteries at the level of head of radius.
b) Circumflex humeral arteries contribute to formation of scapular anastomosis.
c) At its origin cephalic vein lies just posterior to the styloid of the radius.
d) Brachial artery is protected by bicipital aponeurosis.
e) Princeps pollicis artery arises from superficial palmar arch.

228) T/F regarding the blood supply of the upper limb

a) Superior ulnar collateral artery accompanies the ulnar nerve.
b) Profunda brachi artery sends braches on either side of the intermuscular septum.
c) Scapular anastomosis fails to provide adequate blood supply to the distal part of upper limb if the third part of the subclavian artery is blocked.
d) Anterior interosseous artery gives nutrient branch for radius

229) Most of the arteries in the axilla are accompanied by nerves. Which of the following pair correctly states the nerve & accompanying artery?

a) Anterior circumplex humeral / axillary nerve
b) Lateral thoracic artery / lateral pectoral nerve
c) Subscapular artery / lower subscapular nerve
d) Thoraco acromial / intercostal brachii
e) Thoracodosal artery / nerve to lattisimus dorsi

230) Contents of principal neurovascular bundle below the insertion of coracobrachialis

a) Musculocutaneous nerve
b) Median nerve
c) Cephalic vein
d) Brachial artery
e) Radial nerve

231) T/F of principle neurovascular bundle of arm

a) Located in the posterior compartment of the arm
b) Basilic vein is a content
c) Radial nerve leaves the neurovascular bundle first
d) Ulnar nerve leaves the neurovascular bundle by piercing the medial intermuscular septum
e) Brachial artery is palpable throughout

232) T/F regarding the arm,

a) Ulnar nerve pierces the medial intermuscular septum along with the superior ulnar collateral artery.
b) The radial nerve branch to the medial head of the triceps muscle arises at the axilla.
c) Cutaneous supply of the upper part of the deltoid muscle is carried by supraclavicular nerve.
d) Biceps brachii acts as a flexor of the elbow joint when supinated position.
e) The nutrient artery of the humerus arises from the profunda brachii artery.

Arteries of the Upper limb

233) Axillary artery

a) Begins at the posterior border of the first rib.
b) Pass posterior to pectoralis minor.
c) Lies lateral to medial cord in 2nd part.
d) Ligation between subscapular and profunda brachial arteries causes ischemia of the forearm.
e) Lies lateral to the short head of triceps.

234) The brachial artery

a) Lies medial to axillary vein.
b) Superficial throughout the arm.
c) Lies medial to humerus.

d) Begins at the inferior border of the teres major.
e) Ends at the level of the neck of the radius.

235) The radial artery

a) Enters the palm between the two heads of abductor pollicis.
b) Is anterior to the biceps tendon in the cubital fossa.
c) Is accompanied by the radial nerve in the lower third of the forearm.
d) At the wrist lies superficially between tendons of flexor carpi radialis and brachioradialis.

236) The ulnar artery

a) Is the artery of hand.
b) Origin at the level of the head of the radius.
c) Supply ulnar recurrent to anastomose with ulnar collateral branch of the brachial art.
d) Become superficial palmar arch on flexor retinaculum.
e) The pulps of the fingers are supplied by the proper palmar digital arteries.

237) T/F regarding ulnar artery

a) Course in between 2nd and 3rd metacarpal bones.
b) Main contributor of the deep palmar arch.
c) Mark no contribution to superficial palmar arch.
d) Give rise to the proper digital arteries to digit 2-5.
e) Passes between two heads of adductor digiti minimi.

238) Posterior interosseous artery

a) Anastomosis with anterior interosseous artery.
b) Accompanied with posterior interosseous nerve throughout its course.
c) Runs in between supinator and abductor pollicis longus muscle.
d) Pierce in the interosseous membrane.
e) Gives of interosseous recurrent artery.

239) T/F regarding the arteries of the hand

a) The deep palmar arch is mainly formed by radial artery.
b) The deep palmar arch curve medially beneath the long flexor tendons.
c) The superficial palmar arch is a direct continuation of the ulnar artery.
d) The superficial palmar arch communicates with the deep palmar arch.
e) Princeps pollicis artery is a branch of radial artery.

240) Regarding the arterial supply of the upper limb

a) Axillary artery begins at the medial border of the 1st rib.
b) Radial artery begins at the neck of the radius.
c) Radial artery is larger than the ulnar artery.
d) Radial artery passes deep to the adductor pollicis longus.
e) The median nerve crosses the ulnar artery superiorly.

241) Palpable arteries in the upper limb

a) Radial artery
b) Ulnar artery
c) Posterior interosseous artery
d) Brachial artery
e) Profunda brachii artery

242) True/False

a) Brachial artery divides into terminal branches immediately above the elbow joint.
b) Posterior interosseous artery supplies the dorsum of the hand.
c) There is anastomosis between brachial, ulnar and radial arteries around the elbow.
d) Brachial artery can be compressed against the humerus in its entire course in arm.
e) The nutrient artery to humerus from the brachial artery is an end artery.

243) Collateral blood supply to the distal part of the hand can be maintained if the following arteries are occluded

a) 3rd part of subclavian artery.
b) 2nd part of axillary artery.
c) Radial artery at the wrist.
d) Digital arteries at the middle finger.
e) Brachial artery at the cubital fossa.

244) True/False

a) Subscapular artery gives branches to subscapular fossa.
b) Dorsal scapular artery lies along the lateral border of scapula.
c) The supply of circumflex scapular artery is limited to infraspinatus fossa.
d) Dorsal branches of intercostal arteries take part in scapular anastomosis.
e) Branches of the subclavian artery anastomoses with branches of the axillary artery

245) True or False

a) Ulna artery is the principle blood source of the hand.
b) Pulsation of brachial artery can be felt at the medial border of the biceps brachii.
c) Brachial artery crosses the median nerve at the insertion of the coracobrachialis.
d) If brachial artery is ligated in the distal 3rd of arm, blood could still reach the forearm.
e) Brachial artery divides into ulnar and radial arteries at the line of elbow.

246) T/ F regarding the arterial supply of the forearm

a) Interosseous recurrent artery supplies the elbow joint
b) Posterior interosseous artery laterally related to the deep branch of the radial nerve
c) Ulnar artery is the main blood supply to the forearm muscles
d) Radial artery passes deep to the tendon of abductor pollicis longus
e) Pulsation of the radial artery is felt in anatomical snuffbox

247) Ulnar artery

a) Is larger than radial artery
b) Begins at the level of the neck of the radius
c) Passes inferior to the flexor retinaculum
d) Form deep palmar arch
e) Lies lateral to the ulnar nerve in the distal part of the forearm

248) T/F regarding arterial supply for the forearm

a) Ulnar artery passes deep to the pronator teres muscle
b) Carpal bones are supplied its blood supply by posterior interosseous artery
c) Main artery supply to the forearm muscle by radial artery
d) Ulnar artery lies lateral to the ulnar nerve at wrist
e) Radial artery passes superficial to the flexor retinaculum

249) Radial artery

a) Originates in the level of lateral epicondyle.
b) Gives anterior interosseous artery.
c) Accompanied with median nerve.
d) Is felt between flexor carpi radialis longus and brachialis.
e) Forms deep palmar arch.

250) Ulnar artery,

a) a. Passes between forearm muscles that arise from common flexor origin
b) b. Passes deep to pronator teres
c) c. It lies medial to ulna nerve at wrist
d) d. Anastomoses with superficial radial artery
e) e. Supply medial 3 ½ fingers

251) True or false regarding the axillary artery,

a) It originates from the inner border of the first rib
b) it lies outside the axillary sheath
c) Thoracoacromial artery arises deep to the pectoralis minor muscle
d) Subscapular artery is the largest artery
e) Three cords of brachial plexus lie poster to the axillary artery in its' first part

Scapular anastomosis

252) True/False of scapular anastomosis?

a) Scapular artery travels under the suprascapular ligament of scapula.
b) Dorsal scapular artery anastomoses with thoracodorsal artery.
c) Anterior circumflex humeral artery pan through the upper triangular space.
d) Circumflex scapular artery is a branch of profunda brachii artery.

e) If the 1st part of axillary artery is blocked, brachial artery is supplied by suprascapular & dorsal scapular arteries.

253) Arteries of the scapular anastomosis?

a) Thoracoacromial artery
b) Suprascapular artery
c) Thoracodorsal artery
d) Subscapularis artery
e) Superficial branch of transverse cervical artery

254) Arteries that contribute to form the anastomosis around scapula

a) Superior thoracic artery
b) Subscapular artery
c) Suprascapular artery
d) Superficial branch of transverse cervical artery
e) Thoracodorsal artery

255) Scapular anastomosis is contributed by the,

a) deep branch of transverse cervical artery
b) suprascapular artery
c) circumflex scapular artery
d) posterior intercostal artery
e) thoracodorsal artery

256) Blood vessels forming the scapular anastomosis include?

a) Deep branch of transverse cervical artery
b) Suprascapular artery
c) Circumflex scapular artery
d) Post intercostal arteries
e) Sup thoracic artery

Veins of the Upper limb

257) Basilic vein

a) Form by the union of dorsal venous plexus of ulnar side.
b) Is deep to the branch of medial cutaneous nerve of forearm at the elbow.
c) Is subcutaneous until reach to the axilla.
d) Join with cephalic vein by median cubital vein.
e) At the mid arm pierces the deep fascia.

258) Cephalic vein

a) Originate at the medial end of the dorsal venous arch.
b) Valve less.
c) Pierce deep fascia at the cubital fossa.
d) Connect with brachial vein.

e) Used in cannulation.

259) Regarding the cephalic vein

a) Is the largest superficial vein in the body.
b) Terminate in axillary vein.
c) Branching constantly varies from person to person.
d) Origin at the ulnar end of the dorsal venous arch.
e) Contribution branches vary from person to person.

260) Median cubital vein

a) Tributary of the basilica vein.
b) Lies in superficial fascia.
c) Has valves.
d) Separated from brachial artery by bicipital aponeurosis.
e) Is a convenient site for vein puncture.

261) The Median cubital vein

a) Runs upwards and medially to join the cephalic vein.
b) Is separated from the brachial artery by the bicipital aponeurosis.
c) Communicates with the deep veins of the forearm.
d) Not present in all subjects.
e) Is a communication between cephalic and basilic veins.

262) Regarding the veins of the upper limb

a) Cephalic vein origins just lateral.
b) Basilic vein accompanies with the medial cutaneous nerve of forearm.
c) The basilic vein pierces the deep fascia at the level of coracobrachialis insertion.
d) The cephalic vein drains into the axillary vein.
e) The medial cubital vein is separated from underlying artery by an aponeurosis.

263) Regarding upper limb veins

a) Basilic vein pierce the deep fascia in infraclavicular fossa.
b) Basilic vein accompanies with medial cutaneous nerve of the forearm.
c) Cephalic vein runs in posteromedial aspect of forearm.
d) Medial cubital vein is separated by its underlying brachial artery from bicipital aponeurosis.
e) Cephalic vein lies anterior to radial styloid process.

264) T/F regarding the venous drainage of the upper limb

a) Arrangement of the superficial veins are variable
b) Superficial veins contain valves
c) Cephalic and basilica veins are united by median cubital vein
d) Cephalic drain into the brachial veins
e) Basilic vein is in the lateral side of the arm

Axilla

265) T/F regarding the axilla

a) Inferior part of the posterior wall is formed by subscapularis muscle.
b) Its apex is medially bounded by the first rib.
c) It contains the trunks of the brachial plexus.
d) Lateral group of axillary lymph nodes lie deep to pectoralis major.
e) It is the site of damage of the radial nerve in “crutch palsy”.

266) Regarding the axilla

a) Has a medial wall extending vertically from 1st to 3rd ribs.
b) Has soft fat devoid of lymph channels.
c) Axillary nerve passes between subscapularis and teres major.
d) Roots of the median nerve cross the axillary artery.
e) Roots, trunks and divisions of brachial plexus are palpable.

267) Axilla

a) Anterior axillary fold consist of two pectoral muscles and subscapularis.
b) Contain apical lymph nodes draining into right subclavian lymph trunk.
c) Lateral angle covered by the upper parts of biceps and coracobrachialis muscle.
d) When ligated between origins of the subscapular artery and the profunda brachii arteries in the arm will result in ischemia in forearm.
e) Continuous with the triangle of the neck.

268) Regarding the axilla

a) Medial wall is formed by upper 4 to 5 ribs.
b) Clavipectoral fascia is pierced by medial pectoral nerve.
c) Lateral group of lymph nodes lie along the lateral side o axillary vein.
d) Radial nerve gives posterior cutaneous nerve of the arm in the axilla.
e) Second part of axillary artery is posteriorly related to the shoulder joint.

269) Axilla

a) Anterior axillary fold consists of two pectoral muscles and subscapularis.
b) Contains apical lymph nodes draining into right subclavian lymph trunk.
c) Lateral angle covered by the upper parts of biceps and coracobrachialis muscle.
d) When ligated between origins of the subscapular artery and the profunda brachi arteries in the arm will result in ischemia of forearm.
e) Continues with the triangle of the neck.

270) T/F regarding axilla

a) Long thoracic nerve lies on the posterior wall.
b) Long thoracic nerve lies on the lateral wall of the axilla.
c) The central group of axillary lymph nodes lies deep to pectoralis minor.
d) The clavipectoral fascia lies posterior to the neurovascular bundle.
e) Axillary vein lies within the axillary sheath

271) True or false regarding the axilla

a) Anterior border of the apex is demarcated by the lateral border of the 1st rib
b) Lateral thoracic vessels lie in the anterior wall
c) Lateral thoracic nerve lies in the medial wall
d) Trunks of the brachial plexus lies within the axilla
e) Intercostal brachial nerve gives the cutaneous supply to the floor

272) The axilla

a) Medially bounded by the 1st rib.
b) Pectoral group of axillary lymph nodes lie deep to the pectoralis major muscle.
c) Is the site of origin of the internal thoracic artery.
d) Contains the axillary vein within the axillary sheath.
e) Is a site of radial nerve get damage due to a pressure of crutch.

273) The axilla has

a) An apex which communicates with the posterior triangle of the neck.
b) An apex bounded, in part by the medial third of the clavicle.
c) A narrow lateral wall.
d) A posterior wall formed by serratus anterior.

274) Axillary lymph nodes receive lymph from

a) Medial quadrant of the breast.
b) Skin above the umbilicus.
c) Anterior intercostal spaces.
d) Posterior intercostal spaces.

e) Thumb.

275) Regarding the lymph nodes of the axilla

a) Pectoral group lies along the lateral thoracic artery.
b) Enlarged apical lymph nodes cannot be palpated.
c) Deep and superficial parts of the hand are drain in to the lateral group.
d) Lateral group lie medially along the axillary vein.
e) Subscapular nodes lie in the axillary tail.

276) T/F regarding axillary nodes

a) Drain the lymph vessels of the trunk from the level clavicle to umbilicus.
b) Lie deep to the pectoralis major.
c) Lymph of upper limb directly drains to the apical nodes.
d) Lateral group of axillary nodes lie along the subscapular vessels.
e) Swelling can be occurred when axillary nodes are removed.

277) WOF are correct about lymph drainage of axilla?

a) Central lymph nodes drain the back
b) Apical nodes locate at the outer border of first rib
c) Central nodes locate along the axillary vein
d) Posterior nodes locate on the subscapularis muscle
e) Apical lymph nodes drain into subclavian lymph trunk directly

278) T/F regarding axilla,

a) The lateral wall is formed by the intertubercular groove of the humerus.
b) Long thoracic nerve is presented in its medial wall.
c) The pectoral group of nodes lies along the lower border of pectoralis minor.
d) The axillary artery is a continuation of the Subclavian Artery
e) The thoracoacromial branch is the largest branch of the axillary artery.

Cubital fossa

279) The cubital fossa

a) Is a quadrilateral space situated in front of the elbow joint.
b) Is floored by the bicipital aponeurosis.
c) Contains the median nerve.
d) Contains the radial nerve.
e) Is crossed by the medial cutaneous nerve of the forearm.

280) In the cubital fossa

a) Radial nerve innervates the extensor carpi radialis longus muscle.
b) The median cubital vein is separated from the underlying neurovascular structures by the anconeous muscle.
c) Ulnar artery gives rise to common interosseous artery.

d) Median nerve gives branch to pronator teres muscle.
e) The ulnar artery is separated from the median nerve by the deep head of pronator teres.

281) Cubital fossa

a) Biceps forms the floor of it medially.
b) Most medial structure is the median nerve.
c) Ulnar nerve is a content of cubital fossa.
d) Presence of bicipital aponeurosis protects the neurovascular bundle from injuries in venepuncture.

282) T/F regarding cubital fossa

a) Flexor carpi ulnaris is at medial border.
b) Roof is formed by bicipital aponeurosis.
c) Supratrochlear lymph node is present in the cubital fossa.
d) Venae punctures can be done in the cubital fossa.
e) Has the radial nerve deep to the ulnar nerve.

283) A nurse was ordered to take a blood sample from a patient. She isolated the median cubital vein of the right side and extracted the blood sample. What structure aided in preventing any complication during procedure?

a) Bicipital aponeurosis
b) Brachial artery
c) Brachioradialis
d) Pronator teres
e) Tendon of biceps

284) T/F regarding the contents of the cubital fossa

a) Tendon of bicep brachii
b) Brachial artery
c) Ulna artery
d) Ulna nerve
e) Bicipital aponeurisis

285) Bicipital aponeurosis

a) Is an extension of the biceps brachi muscle
b) Blends with deep fascia of forearm.
c) Passes deep to brachial artery.
d) Is superficial to the median nerve
e) Separates the median nerve from the medial cubital vein

286) True or false regarding cubital fossa

a) Medially bound by medial border of pronator teres
b) Radial nerve is a content
c) Lateral cutaneous nerve for forearm lies in the roof
d) Floor is formed by biceps
e) Safe site for venae puncture

287) T/F regarding the cubital fossa

a) The medial border is formed by the brachioradialis
b) The floor is formed by brachialis
c) Brachial artery gives off its terminal branches within it
d) Ulnar nerve is a content
e) It is a preferable place to obtain a blood sample

288) True or false regarding cubital fossa,

a) It is medially bounded by medial border of pronator teres
b) lateral border is formed by medial margin of brachioradialis
c) Bicipital aponeurosis forms the floor of the cubital fossa
d) Median cubital vein is a content
e) Brachial artery is medial to the median nerve in the fossa

289) True or false regarding cubital fossa,

a) The floor is contributed by the supinator muscle
b) Bicipital aponeurosis reinforces the roof
c) It is medially bounded by medial border of pronator teres
d) It contains posterior interroseous nerve
e) Deep branch of pronator teres separates the median nerve from ulnar artery

Carpal Tunnel

290) The carpal tunnel

a) Contains the tendons of flexor digitorum profundus in one plane.
b) Has its anterior wall giving origin to opponens pollicis.
c) Transmits the fibrous flexor sheath and four separate tendons of flexor digitorum superficialis.
d) Has the tendons of flexor carpi radialis and Palmaris longus superficial to it.
e) Extend distal to superficial palmar arch one inch above the wrist joint.

291) The carpal tunnel

a) Formed by the carpal bones and the flexor retinaculum.
b) Which transmit the two sets of flexor tendons in a common synovial sheath.
c) This conducts the ulnar nerve and artery to the palm.
d) Giving passage to the radial artery to complete the palmar arch.
e) Which conducts the median nerve to the palm posterior to the flexor tendons.

292) What lies within the carpal tunnel

a) Palmaris longus tendon
b) Ulnar artery
c) Flexor carpi radialis tendon
d) Flexor pollicis longus tendon
e) Radial artery

Hand

293) Regarding the hand

a) Inflammation involve in the ulna side of the palmar aponeurosis cause Dupuytren's contracture.
b) Palmaris brevis is supplied by median nerve.
c) Palmar aponeurosis separates the superficial and deep palmar arches.
d) Dorsal interossei muscles are tested by spread out fingers against the resistance.
e) Deep palmar arch is formed by radial artery.

294) T/F regarding the hand

a) Skin of the lateral 2/3 of the palm is supplied by palmar cutaneous nerve of median nerve.
b) Both aspects of medial 1 ½ fingers are supplied by the ulnar nerve.
c) Median nerve palsy at the wrist results in monkey's hand.
d) Wrist is flexed in Volkmann's contracture.
e) Dupuytren's contracture results from fibrous contraction of fibrous flexor sheath.

295) The palmar aponeurosis

a) Is attached to the flexor retinaculum.
b) Fuses with the fibrous flexor sheaths of the fingers.
c) Which is a part of deep fascia is adherent to the overlying skin.
d) Separates the superficial and deep palmar arches.
e) May produces a claw hand by progressive shortening of its medial part.

296) T/F regarding synovial sheaths

a) Common synovial sheath of flexor tendons extends from the bases to the heads of the metacarpal bones.
b) Common synovial sheath communicate with the sheath of flexor pollicis longus.
c) Medial part of common synovial sheath is in the mid palmar space.
d) Each long flexor tendon in each finger has its own synovial sheath.
e) Synovial sheath of middle finger extending up to deep palmar arch communicate with the common synovial sheath.

297) The palmar ligament

a) Is a strong thick ligament which lies anterior to metacarpophalangeal joint.
b) Passes from the base of the proximal bone to the neck of the digital bone.
c) Forms a part of the socket for the head of the proximal bone.
d) Margins of it continue with the collateral ligament.
e) Gives attachments to the deep transverse metacarpal ligament.

298) Regarding the extensor expansion

a) Kept in place by a fibrous flexor sheath & deep transverse metacarpal ligament.
b) Have very similar insertions to separate phalanges.
c) Extensor pollicis longus and interosseous muscles contribute to it.
d) Each finger has one on its dorsal side with a lumbrical attached to the lateral margin.

299) True or false

a) Skin of the pulp of the finger enclosing the distal phalanx has no subcutaneous fat.
b) 2nd dorsal interosseous is attached to the lateral side of the middle finger.
c) Flexion at the MP joint is lost when long flexor tendons are completely cut.
d) The pulp of the finger has poor blood supply but a rich nerve supply.
e) Lumbricals innervated by the radial nerve lie anterior to deep transverse metacarpal ligament.

Anatomical snuff box

300) T/F regarding anatomical snuff box

a) Is anteriorly bounded by the extensor pollicis longus.
b) Is demarked when the thumb is fully flexed.
c) Has radial styloid process on its floor.
d) Cephalic vein is the most superficial structure passing through it.
e) Radial pulse can be felt at the snuff box.

301) Boundaries of the anatomical snuff box are

a) Abductor pollicis longus
b) Abductor pollicis brevis
c) Extensor pollicis longus
d) Extensor pollicis brevis
e) Extensor indicis

302) A young adult fell on an outstretched hand. There was tenderness in anatomical snuff box. The fracture was not identified earlier by x-ray. What can be the fractured bone? (SBR)

a) Radius
b) Triquetral
c) Scaphoid
d) Lunate
e) Base of the 1st metacarpal bone

303) T/F regarding anatomical snuff box

a) Medially extensor policis longus is a relation
b) Basilic vein on its roof
c) Trapezius lies in its floor
d) Radial pulse can be felt
e) Tenderness over it is a sign of fracture of scaphoid

Flexor retinaculum

304) What are the structures passing below the flexor retinaculum

a) Ulnar dorsal branch.
b) Median nerve.
c) Flexor pollicis longus tendon.
d) Palmar cutaneous branch of radial nerve.
e) Ulnar artery.

305) The flexor retinaculum is attached to the

a) Hamate
b) Scaphoid
c) Pisiform
d) Triquetrum
e) Trapezium

306) Flexor retinaculum

a) Is a part of the deep fascia of the hand.
b) Doesn't give attachment to the palmar aponeurosis of the hand.
c) Is attached to the bony parts which are not palpable.
d) Is processed on its anterior surface by Palmaris longus.
e) Gives attachment to thenar and hypothenar muscles.

307) T/F regarding the flexor retinaculum

a) Medial attachments are pisiform and tubercle of scaphoid.
b) In flexion of the wrist, it acts as a pulley for tendons which passes behind it.
c) The ulnar nerve lies under the flexor retinaculum along the lateral border of the pisiform bone.
d) Median nerve lies under the flexor retinaculum.
e) Proximal end is at the distal wrist crease.

308) Which passes above the Flexor retinaculum

a) Flexor digitorum profundus tendon
b) Palmaris longus
c) Radial bursa
d) Median nerve
e) Flexor pollicis longus

309) Flexor retinaculum

a) Attaches to the radial styloid process.
b) A thickening of superficial fascia.

c) Gives attachments to the opponens pollicis.
d) Has the palmar cutaneous branch of the median nerve deep to it.
e) Flexor retinaculum split vertically in decompression of carpal tunnel.

310) What are the structures pass superficial to the flexor retinaculum

a) Radial artery
b) Ulnar artery
c) Ulnar nerve
d) Palmer cutaneous nerve of median nerve
e) Flexor carpi radialis

Spaces of the hand

311) Regarding spaces of the hand

a) Midpalmar space lies anterior to 3rd, 4th and 5th metacarpals.
b) Thenar space lies anterior to 1st and 2nd metacarpals.
c) The radial bursa extends into forearm deep to flexor retinaculum.
d) There is very little subcutaneous tissue over the palm of the hand.
e) Synovial sheath of 2nd, 3rd, 4th fingers are closed off proximally at the level of metacarpal joints.

312) Regarding spaces of the hand

a) Radial bursa passes behind the flexor retinaculum.
b) Infections of the superficial pulp spaces of the fingers are restricted to the hand.
c) Space of Parona is anterior to the pronator quadrates.
d) Radial and ulnar bursae are never communicated.
e) Mid palmar space is anterior to the adductor pollicis.

313) Spaces of the hand

a) Pulp spaces are situated in front of terminal phalanx.
b) Ulnar bursa can be drained by an incision across medial side of hypothenar eminence.
c) Inflammation of pulp space can result with necrosis of epiphysis of the terminal phalanx.
d) Tenosynovitis is infection of the synovial sheath to the tendon.
e) Infection of the pulp space of the thumb spread to the radial bursa.

314) T/F regarding spaces of the hand

a) The thenar space lies superficial to the 2nd and 3rd metacarpals.

b) The mid palmar space lies behind the flexor tendons.
c) Sepsis in the 1st digital synovial sheath may spread proximally into the forearm.
d) Oedema does not occur dorsum side in palmar infection.
e) Pulp infection result in necrosis of the diaphysis of the bone.

315) T/F regarding the infections of the hand

a) Advanced pulp space infection of 2nd finger spread to the mid palmar space.
b) Untreated infection of synovial sheath leads to necrosis of flexor tendons.
c) Infection is placed at the center at the distal phalanx to drain the pulp abscess.
d) Infection in space of parona doesn't extend beyond proximal edge of pronator quadratus.
e) A fluid collection in the mid palmar space is located anterior to deep palmar arch.

316) Regarding spaces of the hand

a) The space of Parona situated in between pronator quadratus and interosseous membrane.
b) Pulp space infection leads to avascular necrosis of the epiphysis of the terminal phalanx.
c) Web spaces of the hand communicate proximally with the mid-palmar space through the lumbrical canal.
d) Adductor pollicis is located in the thenar space.
e) Third lumbrical is in mid palmar space.

317) Regarding spaces of the hand

a) The thenar space communicates with the subcutaneous tissue between 1st and 2nd fingers.
b) Infection of the pulp of the finger is limited by the fibrous septa.
c) Mid palmar space communicates with the distal end of the forearm deep to long flexors.
d) Pus in the mid palmar space can be drained by an infection in the 1st web.
e) Interosseous muscles form the posterior boundary of the thenar space.

318) Mid palmar space

a) Lies posterior to the ulnar bursa in the palm.
b) Is separated from the 1st and 2nd metacarpus by 1st lumbrical muscle.
c) Is filled with loose connective tissue.
d) Is separated from the thenar space by adductor pollicis.
e) Infections may spread to the space from the fingers along the lumbrical muscles.

319) Mid palmar space

a) Contains adductor pollicis muscle.
b) Infection can spread into the forearm.
c) Contains deep palmar arch.

d) Infections cause swelling of the dorsum of the hand.
e) Is continuous with thenar space.

320) A 25 years old female complained severe pain in the distal phalanx of the index finger. She had prick injury to the distal phalanx 2 days before and diagnosed to having pulp space infection. Describe the anatomical basis of the above condition. (SBR)

a) Obstruction to digital vein causes ischemia to tip of the distal phalanx
b) Tendon sheath infection due to attachment of flexor digitorum profundus tendons to the base of the distal phalanx
c) Avascular necrosis of the tip of the distal phalanx due to the compression of the arteries in the pulp space
d) Severe pain due to compression of the digital nerve
e) Edema of the distal phalanx due to the presence of the distal phalanx

321) True or false about palmar spaces of the hand?

a) Thenar space is proximally closed off by walls of the carpal tunnel.
b) Thenar space contains the first and second lumbricals muscles.
c) The mid palmar space lies posterior to the long fiber tendons of index, middle and ring fingers.
d) Infections of the synovial sheath of the middle finger spread to the forearm.
e) The epiphysis of the distal phalanx receives its blood supply proximal to the facial septa of pulp space.

322) A young man was admitted to the hospital because of the severe pain in his distal phalanx of his finger after a needle prick. What is the anatomical basis for this (SBR)

a) Avascular necrosis of the distal phalanx due to thrombosis of digital arteries
b) Spread of the infection to the dorsal aspect of finger
c) Irritation of digital nerve
d) Compression of digital veins cause to venous obstruction
e) Due to the presence of tightly packed pulp places

Joints of the Upper limb

323) Regarding joints of the upper limb

a) Articular surfaces are covered by fibrous cartilages.
b) Coracohumeral ligament strengthens the shoulder joint capsule above.
c) Distal radio-ulnar joint is a synovial type pivot joint.
d) Fibers of the interosseous membrane radiates medially and downwards.
e) Wrist joint is a synovial type condyloid joint.

324) T/F regarding joints of the upper limb

a) Distal radio-ulnar joint is a pivot joint.
b) Capsule of shoulder joint is weaker anteriorly.
c) Glenoid labrum increase the stability of shoulder joint.
d) The elbow joint is essentially a hinge joint.
e) Transverse ligament along the metacarpophalangeal joints help to make a firm grip.

325) Regarding joints of the upper limb

a) Subscapular bursa lies in between the tendons of supraspinatus and the coracoacromial arch.
b) The tendons of long head of triceps closely attached to the capsule of the shoulder joint.
c) Movements of wrist joint are associated with the action of carpal bones.
d) Posterior dislocation of elbow joint causes complete claw hand.
e) Cavity of inferior radio-ulnar joint does not communicate with wrist joint.

326) True or false

a) Pivot joints are uniaxial.
b) Bony fragments are completely separated in green stick fracture.
c) In endochondral ossification, mesenchyme calcifies directly.
d) Nutrient artery is directed away from the growing end.
e) Small amounts of movement of secondary cartilaginous joints.

327) Which of the following is correctly matched regarding the joint & its type?

a) Proximal Radio Ulnar Joint – Pivot
b) 1st Metacarpophalangeal Joint – Saddle
c) Sternoclavicular Joint - Secondary Cartilaginous
d) Shoulder - Ball & Socket.
e) Interphalangeal Joints – Condyloid

Shoulder joint

328) Movements of shoulder joint

a) Flexion and extension take place in a plane parallel to surface of the glenoid cavity.
b) Medial and lateral rotation best demonstrated in flexed elbow.
c) Movements of scapula are increased with the movement of the shoulder griddle.
d) Teres major is a medial rotator.

e) Scapula rotates at 90^0 in abduction.

329) WOF are correct regarding the shoulder joint

a) Long head of biceps is intrasynovial.
b) Subscapular bursa communicates with the joint.
c) Stability of the shoulder joint mainly depends on the surrounding muscles.
d) Capsule of the shoulder joint does not extend to diaphysis.
e) Inflammation of the tendon of the infraspinatus causes painful arc syndrome.

330) Regarding the shoulder joint

a) Communicate with outline bursae through capsular openings.
b) Has the most free and varied movements of all joints which are always associated with movements of scapular.
c) Has a lax capsule reinforced by muscles of rotator cuff including supraspinatus.
d) Head of the humerus is directed medially and backwards.
e) Downward dislocation damages the axillary nerve weakening abduction of the arm.

331) Regarding the shoulder joint

a) During climbing to a coconut tree, latissimus dorsi muscle does not contract.
b) Tendon of long head of biceps is extrasynovial and intracapsular.
c) Supraspinatus tendon is separated from the acromion process by the subacromial bursa which continuous with the shoulder joint through a constant opening.
d) Inferior dislocation is common because the capsule of the joint is lax inferiorly.
e) The capsule is attached between the articular margin of glenoid cavity and the supraglenoid tubercle.

332) T/F regarding the shoulder joint

a) Glenohumeral ligaments are the thickenings of capsule.
b) The long head of the biceps lies within the synovial membrane.
c) The joint cavity communicates with subscapular bursa.
d) It normally dislocates posteriorly.
e) It supplied by lateral pectoral nerve.

333) Regarding the shoulder joint

a) The muscles of the rotator cuff arising from the scapula closely attached to the humeral attachments of the capsule.
b) Coracohumeral and glenohumeral ligaments pass from the root of the coracoid process to the greater tubercle.
c) Freedom of movement is dependent upon the muscles attachment to the capsule.
d) The capsule extent to the medial side of the surgical neck of the humerus completely enclosing the upper humeral epiphysis.
e) The subscapularis tendon lies anteriorly.

334) Lateral rotation of the upper arm at the glenohumeral joint

a) Is produced by the contraction of subscapularis and teres major muscle.

b) Is associated with the abduction of the upper arm helped by pectoralis minor.
c) Is produced by the muscle supplied by the 5th and 6th cervical nerves.
d) Is produced by the contraction of infraspinatus which is the chief adductor too.
e) Is produced by the contraction of the anterior fibers of deltoid.

335) Flexion of the upper limb at the shoulder through 180^0 from the anatomical position

a) Requires the contraction of the sternocostal head of the pectoralis major muscle.
b) Involves the contraction of anterior fibers of deltoid.
c) Involves a muscle supplied by the musculocutaneous nerve.
d) Involves the contraction of the serratus anterior muscle.
e) Involves the muscles supplied mainly by the 8th cervical spinal nerve.

336) Regarding shoulder joint

a) Coraco-humeral ligaments are the thickenings of the anterior side of the fibrous capsule.
b) Subacromial bursa communicates with the shoulder cavity.
c) Inferior part of the capsule is the weakest area.
d) Acromion is the most lateral bony point in anterior dislocation.
e) The main stabilizing factor is the extrinsic ligament of shoulder joint.

337) Shoulder joint is more tend to dislocate anteriorly than posteriorly. What is the anatomical basis of this? (SBR)

a) All rotator cuff muscles are originated from the scapula.
b) Capsule of shoulder join is attached to shaft of humerus.
c) Long head of biceps is superior to the shoulder joint.
d) Coracoclavicular ligament prevent the anterior dislocation of the humerus.
e) Joint capsule is lax inferiorly.

338) Regarding the shoulder joint

a) Supplied by the axillary nerve
b) Shoulder joint capsule is lax/deficient anteriorly
c) Short head of biceps is in the joint capsule
d) Subscapular bursa communicate with the shoulder joint
e) Glenoid labrum injury occur in posterior dislocation

339) 32 year old man fallen out stretched hand present to emergency department with a deformity on right shoulder. X-ray shoulder joint has revealed

on anterior dislocation with humeral head in subcoracoid position. Anatomical basis for position of humeral head?

a) Action of adductors of shoulder on humeral head.
b) Action of lateral rotators of shoulder on humerus.
c) Articulation of the humeral head to shallow glenoid labrum
d) Capsule is lax anteriorly
e) Inferiorly reduced stability due to deficiency of rotator cuff muscles

340) Regarding shoulder joint

a) Capsule is lax inferiorly
b) short head of biceps lies within the fibrous capsule.
c) Subcapsular bursa communicate with the joint cavity
d) It is a complete ball and socket joint
e) The whole abduction of the joint is done by the deltoid muscle

341) True or false regarding shoulder joint,

a) Head of humerus is directed postero medially
b) A part of the capsule is attached to the diaphysis of the humerus
c) Lateral epicondyle is a part of the capsule
d) Latissimus dorsi is an extensor
e) Tendon of long head of biceps brachii is extra synovial

Sternoclavicular joint

342) Regarding sternoclavicular joint

a) Is a synovial joint.
b) Arch of aorta is a posterior relation.
c) Sternoclavicular ligaments are thickenings of the capsule.
d) The joint cavity is not divided by an articular disc.
e) Interclavicular ligament is fused with the jugular notch of the manubrium sterni.

343) T/F regarding sternoclavicular joint

a) Is a modified saddle joint which allows movement in right angled axes.
b) Is the only articulation between upper limb and trunk with the fulcrum of movement at the sternal end of the clavicle.
c) Has a single C shaped cavity with a complete articular disc.
d) Is a secondary cartilaginous joint closely related to large blood vessels.
e) Has unequal articular surfaces allowing easy displacement.

344) Regarding sternoclavicular joint

a) It is a synovial joint.

b) Behind the sternoclavicular joint lies the common carotid artery on the left.
c) Owes most of its stability to its capsular ligament.
d) Is an ellipsoid variety.
e) Lies anterior to aortic arch.

345) Sternoclavicular joint

a) Is a synovial plane joint.
b) Contains articular surface is covered by fibrous cartilage.
c) Has a wedge shape articular disc cartilage.
d) It is stabilize by the sternoclavicular ligament.
e) Dislocates anteriorly.

346) Regarding sternoclavicular joint,

a) It is a synovial joint
b) Behind the sternoclavicular joint lies the common carotid artery on the left
c) Owes most of its stability to its capsular ligament
d) Is an ellipsoid variety
e) Lies anterior to aortic arch

Elbow Joint

347) T/F regarding elbow joint

a) The capsule is thin and loose anteriorly.
b) The lateral collateral ligament attaches distally to the annular ligament.
c) Supinator action of the biceps is due to its site of insertion.
d) Elbow joint can be safely approached by a posterior vertical incision.
e) Sudden jerky movement of forearm in a child causes sub luxation of the radial head from annular ligament.

348) The elbow joint

a) Communicate with the superior radio-ulnar articulation.
b) Has a synovial membrane closely adherent to the articular capsule.
c) Has a capsule firmly attached to the articular bones.
d) Is a hinge joint between radius, ulna and humerus but abduction and adduction of ulna can occur.
e) The radial collateral ligament is attached to the neck of the radius.

349) True or false of elbow joint?

a) Joint cavity continues with the proximal radio-ulnar joint.
b) Capsule of the elbow joint is weakened posteriorly.
c) It is supplied by the posterior interosseous nerve.
d) Radial collateral ligament provides origin to extensor carpi radialis brevis.
e) Carrying angle is more prominent in the pronated position

350) 40 year old man present with pain at the right elbow. After falling with out stretched hand on the examination found that triangular bone formation of the elbow joint is not present . Elbow dislocation was diagnosed (SBR)

a) Between lateral epicondyle and lateral supracondylar ridge
b) Between medial epicondyle and the supracondylar ridge
c) Between two epicondyles
d) Between both epicondyles and the coronoid process
e) Between two epicondyles and olecranon process

351) True or false regarding elbow joint

a) Humero-radial joint is a synovial ball and Socket joint.
b) Elbow joint stability is mainly by ligament
c) The main Flexor is Brachialis.
d) Musculocutaneous nerve innervates the joint
e) Radial collateral Ligament is attached to the Annular ligament.

352) T/F regarding the elbow joint,

a) Synovial hinge joint.
b) Formed by articulation of humerus with radius & ulna.
c) Capsule is attached to annular ligament.
d) Collateral ligaments prevent hyperextension of the joint.
e) Student's elbow is caused by inflammation of the olecranon bursa.

Radio-ulnar joint

353) The proximal radio-ulnar joint

a) Is of the condyloid variety.
b) Occurs between the head of the radius and the radial notch of the ulna.
c) Is stabilized mainly by the surrounding capsular ligament of the elbow joint.
d) Owes its stability mainly to the annular ligament.

354) The distal radio-ulnar joint

a) Is a synovial joint of the pivot variety.
b) Owes its stability mainly to the capsular ligament.
c) With the superior radio-ulnar joint allows both supination and pronation to occur.
d) Pronation is a powerful movement because of the action of biceps.

355) Regarding the radio-ulnar joint

a) Pronation assisted by anconeus is more powerful than supination.
b) The axis of movement is a lying passing through the center of the head of the radius and the styloid process of the ulna.
c) Annular ligament and the radial notch of the ulna articulate with the head of the radius which is easily dislocated downward.
d) Weak capsule of the distal joint blends with the interosseous membrane running downwards and medially from the radius to ulna.
e) During pronation the distal end of the radius carries the hand and articular disc with it.

356) T/F regarding interosseous membrane

a) Interosseous membrane is attached distally to the tuberosity of the radius.
b) Prevents movements of distal radio-ulnar joint.
c) Pierced by posterior interosseous artery.
d) Transmit compressed forces radial to ulna.

357) About the proximal radioulnar joint,

a) It's a pivot joint.
b) Strengthen by annular ligament.
c) Cavity does not communicate with the elbow joint.
d) The median nerve innervates the muscles responsible for the pronation of the proximal radioulnar joint.
e) The radioulnar joint can be tested by flexion of the elbow to 90°

Wrist joint

358) Regarding the wrist joint

a) Synovial type ellipsoid joint.
b) Joint cavity communicates with intercarpal joint cavities.
c) The proximal articular surface of scaphoid extent more on the dorsal surface.
d) If the median nerve is damaged in the lower half of the forearm the movements of the joints are normal.
e) Adduction, extension and medial rotation are more than abduction, flexion and lateral rotation respectively.

359) Regarding the wrist joint

a) At rest scaphoid, lunate and triquetrum articulate with the distal end of the ulna.
b) Movements at the radiocarpal joint are always associated with movements at the intercarpal joints.
c) Has a long radius transversely and a short radius antero-posteriorly.
d) In the adducted hand the joint and the triquetrum are separated from the head of the ulna by articular disc.
e) Communicate with the joint cavity of distal radio-ulnar joint.

360) T/F about wrist joint

a) It is condyloid in type.
b) The triangular articular disc separates the distal end of the radius from the joint cavity.
c) Commonest form of dislocation is the posterior dislocation.
d) Collateral ligaments are poorly developed.
e) The range of abduction considerably less than the adduction.

361) T/F about wrist joint

a) Saddle shape joint.
b) Fibrous capsule is attached to the pisiform.
c) Rotation is possible when joint is extended.
d) In adduction, lunate passes laterally and touches the radius.
e) When person falls on an outstretched hand, the force transmits to the radius via all the proximal carpal bones.

362) Regarding the wrist joint

a) It is a condyloid joint.
b) Proximal end is covered by hyaline cartilage.
c) Allow rotatory movements.
d) Radial artery is anteriorly related to it.
e) Communicate with the joint cavity of distal radio-ulnar joint.

363) T/F about wrist joint

a) Articular disc is attached to the styloid process.
b) Fibrous capsule attached to the pisiform.
c) Triquetrum directly articulates with the radius at resting position.
d) Anterior dislocation is more common than posterior dislocation.
e) It is a synovial condyloid type joint.

364) True or false regarding the wrist joint

a) It is a synovial ellipsoid joint
b) Articular disk articulate with the ulnar
c) Hamate articulate with the radius
d) Distal radio-ulnar joint articulate with wrist joint
e) Motor innervation to wrist extension is supplied by C6 spinal segment

365) True or false regarding the wrist joint,

a) It is a synovial ellipsoid joint
b) Distal radio-ulnar joint communicates with it
c) Distal articular surface of the ulna articulates with the triquetrum
d) Flexor carpi radialis is an abductor

Metacarpophalangeal joint

366) Regarding metacarpophalangeal joint

a) Is a freely movable ellipsoid synovial joint permitting active rotation.
b) Has a very thick palmar ligament attached firmly to the neck of the metacarpal.
c) Includes abduction and adduction in the range of movements which is less in the thumb.
d) On the dorsum of the fibrous capsule is replaced by the extensor tendon.
e) The phalanges converge on flexion mainly by the long flexors.

367) The metacarpophalangeal joints

a) The synovial joints of the hinge variety.
b) Of the medial four digits are bound together.
c) May be abducted by the dorsal interossei muscle.
d) May be adducted by the palmar interossei muscles.

Upper Limb Deformities

368) True or False

a) Brachial plexus injuries may occur from traction on the arm during birth.
b) Injury to inferior trunk is associated with 'Horner's syndrome'.
c) Wrist drop result from damage to the deep branch of the radial nerve.
d) Volkmann contracture is a result of contraction of palmar aponeurosis.
e) Division of ulnar nerve leaves a relatively efficient hand.

369) True or False

a) Lower trunk lesion gives rise to claw hand.
b) Lesion of T_1 leads to waste of the hand muscles.
c) Injury to median nerve at the wrist joint can lead to 'ape' like thumb.
d) Injury to radial nerve at the elbow, 'wrist drop' occurs.
e) Ulnar claw hand becomes more prominent when an ulnar nerve lesion occurs at the elbow.

370) True or False

a) Saturday night palsy is due to radial nerve damage in axilla.
b) Loss of skin sensation over the lower half of the arm is due to damage to axillary nerve.
c) Damaged to the ulnar nerve causes 'Winging scapula'.
d) Damaged to the radial nerve in the spiral groove results in wrist drop.
e) Inability to oppose the thumb against the little finer is a feature of median nerve damage.

371) Following a fall injury a patient was admitted to the emergency department. He was diagnosed as the fracture to the middle 1/3 of the clavicle what are the vital structure that can be damaged in this fracture (SBR)

a) 1st part of axillary artery
b) Posterior trunk of brachial plexus
c) Right brachiocephalic artery at the its origin
d) 3rd part of subclavian artery
e) C5 root of brachial plexus

372) Regarding carpal tunnel syndrome

a) Syndrome consists of a burning pain along the distribution of the median nerve.
b) It is supplied by compression of the median nerve within the tunnel.
c) Weakness of adduction of the thumb.
d) No paresthesia occur over the tunnel eminence.
e) Decrease sensation over the lateral 2/3 of the palm.

373) Erb-Duchenne paralysis

a) Abduction of the shoulder joint above 90^0 is loss.
b) Nerve to subclavius is get involved.
c) Forearm becomes pronated.
d) Extensors at the elbow become prominent.
e) Anesthesia occurs over the tip of the shoulder.

374) A patient was admitted to the casualty ward due to a stab injury to the junction between upper & middle one third of the forearm. The anterior interosseous nerve was cut. What can be the problem? (SBR)

a) Lateral deviation of the hand at wrist.
b) Flexion of the thumb
c) Loss of sensation on the anterior aspect of lateral three and half fingers.
d) Loss of sensation over the thenar eminence.
e) Supination is weak.

375) In Erbs' paralysis

a) Flexion at the MP joints is lost.
b) C_5 and C_6 are affected.
c) Arm is medially rotated due to paralysis of deltoid and subscapularis.
d) Forearm is pronated.
e) Opposition of the thumb is weak or lost.

376) What is the most suitable deformity occurs when superficial cut off the flexor retinaculum?

a) Wrist drop
b) Ape like thumb
c) Sensory loss over thenar eminence
d) Abduct of the fingers
e) Flexion of the distal phalanges of fingers

377) Elderly women fallen outstretched hand fracture at the distal radial (Colle's fracture). What is the deformity caused? (SBR)

a) Distal fragment shift posterior and proximally
b) Avascular necrosis of radial head
c) Ulnar styloid extend more distally than of radius
d) Distal end of radius displaced anteriorly
e) Appearance of claw hand

378) 40 year old man presents with features due to superficial cut of his posterolateral aspect of right forearm. On examination his wrist extension is weak. But no sensory loss on hand and forearm. What is the structure damaged? (SBR)

a) Anterior interosseous nerve
b) Deep branch of Ulnar nerve
c) Posterior cutaneous nerve of forearm
d) Posterior interosseous nerve
e) Superficial branch of radial nerve.

379) Ulnar nerve injury at the wrist

a) Sensory loss of the dorsal surface medial 1/3 of hand.
b) Impairment in adduction of the thumb.
c) Prominent claw hand compared to the ulnar nerve injury at elbow.
d) Wasting of thenar muscles.
e) More prominent functional impairment than median nerve injury.

380) Teacher complaining who had a penetrating injury to right forearm before a month is unable to grip a piece of chalk while writing. Flexion of interphalangeal joint of the right thumb is lost. What is the damaged nerve? (SBR)

a) Anterior interosseous nerve
b) Posterior interosseous nerve
c) Median nerve at wrist
d) Ulnar nerve
e) Radial nerve

381) Which of the following pairs are correct according to the nerve injuries

a) Injury to axillary nerve – paresthesia occurs hand breadth below the acromion
b) Paralysis of subscapularis – impaired abduction of arm
c) Damage to supraspinatus muscle inability to abduct the arm

382) Ulnar nerve injury at the elbow

a) Sensory loss occur over hypothenar eminence
b) Less claw hand deformity than injury in wrist
c) Sensory loss in dorsum of 5th digit
d) Paralysis of 2nd lumbrical muscle
e) Impairment of thumb abduction

383) 64 years woman, carpal tunnel syndrome undergone decompression surgery. Superficial structures of flexor retinaculum are damaged. Which deformity can be seen? (SBR)

a) Claw deformity of thumb
b) Loss of sensation in thenar eminence
c) Difficulty in flexing right index finger
d) Weakness of flexion of right index finger
e) Wasting of right thenar muscle

384) A patient was admitted to the hospital complaining pain at shoulder joint. It was observed that arm was adducted, medially rotated and sensory loss over the lower half of the deltoid muscle. The diagnosis was anterior-inferior shoulder dislocation. What was the injured nerve? (SBR)

a) Axillary nerve
b) lateral pectoral nerve
c) Lateral thoracic nerve of forearm
d) Lower subscapular nerve
e) Supraclavicular nerve

385) 70 year old male patient was presented with brachial plexus injury. He was diagnosed with right side lung carcinoma in right lobe with the involvement of neuronal structure related with 1st rib. What is the deformity/ disability? (SBR)

a) Inability to extend the elbow
b) Loss of sensation on the lateral aspect of forearm
c) Loss of sensation over deltoid
d) Weakness in flexion of the elbow joint
e) Weakness of small muscles of the hand

386) Compression of the median nerve at the wrist would results in

a) Loss of sensation over the thenar eminence
b) Paralysis of the first lumbrical
c) Causes paralysis of the abductor pollicis longus muscle
d) Inability to flex the index finger at the distal interphalangeal joint
e) Ape like hand

387) A 35-year-old painter was presented to the surgical clinic with the inability to paint with his right arm for the last two weeks. When taking history, he mentioned about his right shoulder dislocation, reduced a month ago using local anesthesia. On examination, right shoulder abduction impairment was detected. Which of the following muscle is most likely to cause his disability?

a) Deltoid muscle
b) Serratus anterior
c) Subscapularis
d) Supraspinatus
e) Teres minor

388) A 28-year-old woman presented to the surgical clinic with a complaint of retraction of the nipple and thickening of the skin of her right breast. Following a proper examination, it is diagnosed as a carcinoma of the breast. What is the anatomical basis of retraction of the nipple?

a) Infiltration of carcinoma into fat tissues
b) Infiltration of carcinoma into longitudinal tissues of nipple
c) Infiltration of carcinoma into the lactiferous ducts
d) Infiltration of carcinoma into the lobules
e) Infiltration of carcinoma into the suspensory ligaments of the breast

389) In a 60-year-old woman, features of carpal tunnel syndrome were found. What is the most common feature?

a) Sensory loss over the thenar eminence
b) Wasting of hypothenar eminence
c) Numbness over the palmar aspect of the lateral two-and-half fingers
d) Numbness over the medial two and half fingers
e) Weakness of flexion of distal interphalangeal joint on index and middle fingers

390) A 34-year-old man presents with a deformity of his right forearm and hand and is diagnosed as having Volkmann ischemic contracture. He had a supracondylar fracture of the right humerus one year back, leading to this problem. What is the most likely structure injured causing this deformity?

a) Radial nerve at the elbow joint

b) Median nerve at the elbow joint
c) Brachial artery at the elbow
d) Common flexor muscles origin
e) Common extensor muscles origin

391) Which of the following muscles are paralyzed in Erb's palsy?

a) Latissimus dorsi
b) Pectoralis Minor
c) Teres minor
d) Biceps Brachii
e) Deltoid

392) A 24years old man was admitted to the surgical ward following a road traffic accident, on examination round counted on shoulder and loss of sensation over the insertion of deltoid was observed. what is the nerve damaged?

a) Axillary nerve
b) Median nerve
c) Musculocutaneous nerve
d) Suprascapular nerve

393) Years old boy falling from height. He has pain, swelling in elbow with evidence of nerve injury. X ray revealed supracondyler fracture and anterior displacement of proximal fragment. What is the nerve?

a) Medial cutaneous nerve of the forearm
b) Median nerve
c) Musculocutaneous nerve
d) Radial nerve
e) Ulnar nerve

394) A man present to the surgical clinical ward with dysfunction in his right hand after a month of doing a surgery to distress the carpal tunnel syndrome .clinician diagnosed that while this surgery the recurrent branch of median nerve has injured . What is significant that appears in his hand helped that diagnosis?

a) Loss of flexion at the interphalangeal joint of the thumb
b) Loss of flexion at the metacarpophalangial joint of the index finger
c) Loss of the flexion at the metacarpophalangial joint of the thumb
d) Positive froment's test
e) Positive pen touch test

395) A 25 year old man attended the clinic, complaining, weakness of fingers in the right hand. Paralysis of dorsal interosseous muscles was diagnosed. What action of the hand is impaired?

a) Abduction of the middle finger
b) Abduction of the ring finger
c) Abduction of the thumb
d) Adduction of the middle finger
e) Adduction of the thumb

UPPER LIMB REVIEW

Surface Anatomy of Upper Limb

1) T T F F F
- b) *Refer Ellis 165pg (14th Ed)*
- c) Posterior border
- d) Head of the radius
- e) Lower end

2) F F T T F
- d) *Refer Ellis 3pg (14th Ed)*

3) T T F T T
- a) Can be felt at the apex of the axilla with the shoulder abduction. *Ellis 165pg (14th Ed)*
- b) *Refer Cunningham vol 1, 28pg*
- e) *Refer Cunningham vol 1, 66pg*

4) T F T F T
Refer Ellis 168pg (14th Ed)

5) F T T F T
- a) Medial border
- d) Posterior aspect
  Ellis 165pg (14th Ed)

6) T F F T F
- b) Hypothenar eminence
- c) Medial to the ulnar pulse
- e) Surgical neck of the humerus

7) T F T F F
- b) When the arm is abducted
- d) Greater tubercle
- e) *Refer Ellis 165pg (14th Ed)*

8) T – T T T
- e) Styloid process of radius scaphoid and first metatarsal forms the floor of the anatomical snuff box

Bones of the Upper limb

9) T F F T T
- b) Hypothenar eminence
- c) Proximal end, *Ellis 174pg (14th Ed)*

10) F F T T T
- a) Except on the inferior side. *Refer Cunningham vol 1, 77pg*
- b) Heads of the metacarpals
- e) In flexion equilateral triangle

11) T F T F T
- a) *Refer Ellis 177pg (14th Ed)*
- b) Lower end
- d) It takes 14-16 days after injury
  For early diagnosis- ultrasonography
  Gold standard - MRI
- e) Atavistic epiphysis is present as a separate bone in lower animals but in human skeleton it fuses with other bone. Eg: posterior tubercle of talus & coracoid process, *Kulkarni 22pg (2nd Ed)*

12) F T F F F
- a) Pisiform has 2 ossification centers
- b) *Refer Ellis 174pg (14th Ed)*
- c) Proximal end/ base
- d) Growing end fuses with the shaft later
- e) Distal to the ulna

13) F T T F F
- a) Fracture of the clavicle is the commonest
- b) Due to the action of biceps
- c) Dinner fork deformity
- d) Read 11 d review
- e) It may be a cause but not the common cause

Clavicle

14) T T T F T

15) F F T F T
- a) Acromion forms the bony prominence
- b) Divisions of the brachial plexus
- d) Lateral fragment is depressed due to the weight of the limb

16) T F F T T
- b) Has no medullary cavity/ bone marrow
- c) Middle 1/3 of the clavicle
- d) Injury to subclavian vessels is extremely rare. *Refer Ellis 171pg*

17) F T T T T
- b) Anterior axillary wall is made by pec. major, pec. minor, subclavius & clavipectoral fascia
- c) Through costoclavicular ligament

18) F F F T T
- b) There is a secondary centre for medial end. There may be a secondary centre for the acromial end
- c) Backward movement

Scapula

19) T F T T F
- d) Triangle of auscultation is formed by medial border of scapula, superior border of latissimus dorsi, lateral border of trapezius
- e) 1st digitation attaches to the superior angle. 2nd and 3rd digitations attach to the medial border. Last 5 digitation attach to the inferior angle.

20) T T T F T
- d) Teres minor lateral rotation. Teres major medial rotation
- e) Long thoracic nerve supplies serratus anterior

21) T T F T F
- b) Pect minor and major
- c) Inferiorly

22) T F T T F
- b) Reduced
- c) Subscapular bursa
- e) Between two heads of teres minor

23) F T F T F
- a) Gives attachment to pect minor short head of biceps and coracobrachialis
- c) Dorsal surface
- e) Medial border

24) F T F F F
- a) Below
- d) Supraglenoid tubercle
- e) Medial end of clavicle is depressed

25) F T F F T
- a) Coracobrachialis, short head of biceps brachi to the tip of coracoid process (*Refer Snell 9th edi pg 341 fig 9.7)*
- b) *Refer Cunningham 16th edi pg 52*
- c) Inferior glenoid tubercle
- d) Lateral border *(Refer Grants 15th edition pg 94 fig b)*
- e) Laterally medial border of scapula
  Medially lateral border of trapezius
  Inferiorly superior border of latissimus dorsi

26) T _ F F T
- c) 7 secondary centres

d) Anastomosis around elbow as interosseous recurrent artery *(grants 15 edi pg 140)*

27) Answer TTTTF

a)True
b)True-Lateral rotators of the shoulder joint(Infraspinatus,teres minor,posterior fibers of the deltoid) (kuppi note page 11)
c)True-Medial rotators(pectoralis minor,subscapularis,teres major,latissimus dorsi,anterior fibers of the deltiod)
d)True-winging of the scapular is a condition where the medial border of the scapular protrudes away from the thorasic wall.It occurs due to the paralysis of serrates anterior muscle which is a result of damge to the long torasic nerve during mastectomy or etc.
e)False-Rhomboid muscles are supplied by dorsal scapuler nerve.

Humerus

28) F T T T T

a) Surgical neck
e) Upper end

29) F T T F T

a) Posteromedially
d) Upper end

30) T T T T T

Axillary nerve damaged

31) T F T T F

b) Posterior to medial epicondyle
e) Lesser tuberosity

32) T F T T

a) Epiphysis of growing end is 1st to appear and last to fuse
b) Axillary nerve
d) *Refer Snell 9th edi 364 pg shoulder joint*

Radius

33) F F F F T

a) Head
b) Lunate
c) Distal one third palpable *(Grants 15th edi pg 65)*
d) Attached to coronoid process of ulna

34) F F T T

a) Head is situated proximally
b) Ulna gives attachment

35) F F T F T

a) Posterior part of tuberosity
d) Pivot joint

36) _ T F T F

c) Only proximal part *(refer grants 15th edi pg 72)*

Ulna

37) T T F T F

c) Triquetrum attaches with articular disk only in adduction of wrist joint

38) T F T T F

b) Posterior surface

Carpal bones

39) T T F F T

c) Scaphoid or trapezium
d) Concave

40) F F T F

a) Only proximal and distal
b) Scaphoid, lunate, triquetral
d) Lumbricals ,origin- tendon of flexor digitorum profundus, insertion-extensor expansion of medial four fingers

Ossification of bones of the upper limb

41) T T F F F

a) One primary ossification centre and seven secondary ossification centres
c) Greater in adults
d) 3 months
e) Synovial joint

42) F T T T T

a) 8th week
c) Upper end has three secondary centre appears at 1 year,2 year,5 year

43) T F F T F

b) Heads of phalanges
c) At bases

44) F F T T T

a) At 6th year
b) 5th year
e) scaphoid-6th year
lunate- 1st year
triquetral- 1st year
pisiform-12th year
trapezium-6th year
trapezoid-6th year
capitate-3rd year
hamate-2nd year

45) T F F T _
b) after birth
d) 18-20

Pectoral Region

46) T F T F F
a) Lateral pectoral nerve-pec.maj. medial pectoral nerve-pec.maj and pec.min.
b) structures that pierce clavipectoral fascia
C- Cephalic vein
A-Acromiothoracic artery
L- Lateral pectoral nerve
L- Lymphatics
d) musculocutaneous nerve
e) surgical neck of humerus

47) T F T T T
b) superior
e) 60% upper lateral quadrant

48) T F F T T
a) Structures that pierce clavipectoral fascia
C-cephalic vein
A-Acromiothoracic artery
L-lateral pectoral nerve
L-lymphatics
b) Continuation of subscapular artery
c) Along axillary vein

49) T T T T F
b) Read 47 a. review
c) Structures in anterior axillary wall-pec.maj., pec.min., subclavius, clavipectoral fascia
e) Floor of axilla is made by axillary fascia

50) Review FFTTT
"CALL"- cephalic vein, acromiothoracic/thoracoacromial artery, lateral pectoral nerve, lymphatics.

51) Answer FTFTT
a)False
b)True
c)False
d)True
e)True- Structures that pierce the clavipectoral fascia
C-cephalic vein
A-acromio thorasic arter
L-lateral pectoral nerve
L-lymph vessels

Mammary Gland

52) T F T T F
b) In front of deep fascia
d) In lactiferous sinuses
e) Internal thoracic nodes

53) T F T T T
b) Modified apocrine gland
d) 75%
e) Dimpling of the skin over a carcinoma of the breast results from malignant infiltration and fibrous contraction
Refer Ellis 14th edition page 214

54) F T T T T
a) 3rd rib

55) F T T T F
a) Deep fascia lies behind the mammary gland
c) Internal thoracic and parasternal nodes

56) F T T T T
b) Perforating branches of internal thoracic art., intercostal arteries, lateral thoracic artery, thoracoacromial artery
d) Anterior axillary lymph nodes

57) T T T T T

58) F T T T T

a) Axillary tail extend upward and laterally pierces the deep fascia at the lower border of the pec. Maj. and enters the axilla
e) *Further reading Snell page 336*

59) T F T T T

a) 1/3 of breast lies over SA muscle.
b) Nipple lies in the fourth intercostal space/ 4th and 5th ribs.
d) To avoid cutting lactiferous ducts.
- *Ellis' C.A. (14th Ed)*

60) T T F T T

a) From the milk line appearing on the chest wall as an invagination of the surface ectoderm.
c) Drain to the deep lymphatics with the subareolar lymph nodes of the skin
e) Suggests of a carcinoma involving the milk ducts.

61) Answer TFTFT

a)True-brest is a modified apocrine sweat gland and it has an ectodermal origin.
b)False-Horizontaly it extends from lateral border of the sternum to the mid axillary line.
Vertically:from 2nd to 6th rib
c)True-Lateral quadrants:Drain in to anterior axillary or pectoral lymph nodes(about 75%)
Medial quadrants:20% drain into parasternal lymph nodes via vessels which are lying along the tributeries of internal carotid artery
5% drains to posterior intercoastal nodes(snell 9th edi,page 337)
d)False-Montgomery glands are sebaceuos glands.They lie in the areola.They lubricate the areola and also the may form sebaceuos cyst which may in turn become infected.
e)True

62) T T F T T

b) 60% of the breast carcinomas occur in the supero-lateral quadrant.

c) Retromammary space lies between the base of the breast and the pectoral fascia, filled with loose areolar tissue.

63) F T F T T

a) Sweat glands and sebaceous glands are primarily cutaneous glands but mammary glands are modified sweat glands embedded in the subcutaneous fat tissue.

b) If the tumor is recent, inversion will reflect it.

d) Cooper ligaments extend from subcutaneous tissues to the fascial layer of the chest wall.

e) Due to lymph edema of the skin due to underlying tumor. *(Snell's C.A.)*

64) F T F F F

a) A modified sweat gland

c) Supplied by Anterior perforating branches of the internal thoracic artery, Lateral mammary branches of the posterior intercostal arteries, lateral thoracic and acromiothoracic branches of axillary artery.

65) T F T T F

a) Overlies 2-6 ribs, 2/3 of breast lies on pectoralis major and 1/3 on serratus anterior.

b) Superficial to pectoral fascia.

e) Due to fibrous contraction of milk ducts.

66) F T T T T

b) Attached by ligaments of Cooper

c) Montgomery glands are large modified sebaceous glands which lubricate the areola.

d) By lateral perforating branches (lateral mammary arteries) - *Ellis' C.A. (14th)*

67) T F T T T

a) Under influence of ovarian hormones ducts elongate but gland and enlarges due to deposition of fat. - *Snell's C.A.*
b) Each lobule opens by its lactiferous duct on the nipple.
d) By the milk line of surface ectoderm

68) T T T T T

a) 75% of the total lymph drainage of breast
b) Middle part drains to parasternal nodes and posterior intercostal nodes.
c) Along the tributaries of internal intercostal vessels.

69) F T F T F

a) Extends vertically from 2nd rib to 6th rib and horizontally from lateral sternal margin to mid axillary line.
b) Pass upwards and laterally
c) Only 15-20 lobes are present. Number does not increase, just the
size during pregnancy and lactation.
e) Extend up to deep fascia.

70) F T F T T

a) Doesn't lie directly on intercostal muscles but, pectoralis major and serratus anterior.
b) The plexus of Sappey pass to the deep lymphatics of the breast.
c) Open in the axillary tail. Abdominal Scarpa's fascia to form a posterior capsule (*Last's anatomy)*

71) T T T T F

Lymph from the breast drain to,

01. Anterior axillary nodes (75%)
- Ipsilaterally and contralaterally
02. Infraclavicular nodes to the Apical Axillary Nodes
03. Parasternal nodes (internal thoracic nodes)
04. Posterior intercostal nodes. (*Ellis C.A-14th)*

72) T T T T F

Breasts receive blood from,

01. Lateral thoracic artery
02. Acromiothoracic artery
03. Superior thoracic artery
04. Perforating branches of internal thoracic artery giving the internal mammary artery
05. Lateral perforating branches of posterior intercostal arteries.

73) T T T F F

a) Lactiferous sinus lies beneath the areola. *(Kulkarni's C.A. - 2nd Ed)*
b) As the upper quadrants are more tend to develop breast carcinomas
c) This lower ligaments help to anchor the gland and connect the base of the gland to pectoral fascia, without hampering the mobility of the normal gland on the contracted Pec. Major muscle. (*Kulkarni)*

74) F T T F T

a) Situated on it

75) TTTTT

Venous drainage of breast is corresponding to its arterial supply.

76) FFTTT

a. It is modified apocrine sweat gland.
b. Lies in superficial fascia of the pectoral region.

d. Therefore upper lateral quadrant is more prone to carcinomas.

Muscles of the upper limb

77) T F T F T

a) Trapezius is the muscle involved in elevation of the arm over the head which was initiated by supraspinatus. Trapezius is innervated by accessory nerve.
b) Subscapularis is a lateral rotator of the arm and a shoulder joint stabilizer.
c) Anconeus holds the ulna on humerus and abducts the ulna.
d) Biceps brachii is a stronger supinating muscle. *(Cunningham -16th Ed.)*

78) T F T F T

79) F T F T T

a) Serratus anterior origins from upper 8 ribs.
b) Actions of serratus anterior are protraction of entire scapula and holding against the thoracic wall and lateral rotation of the scapula. Paralyzing of the muscle is presented as winging of the scapula.
c) Suppled by the long thoracic nerve. Nerve roots are C5, C6 and C7.
e) Pronator teres is supplied by median nerve directly and pronator qudratus is supplied by anterior interosseous branch of median nerve. *(Snell's CA - 10th)*

80) T F F T T

a) Actions of deltoid: Abduction of arm, flexion and medial rotation, extension and lateral rotation.
b) Supplied by lower subscapular nerve.
d) Dorsal scapular nerve innervates both rhomboid muscles and levator scapulae.
e) Protraction of scapula is done by serratus anterior, pectoralis major and minor.

81) T T T T F

Medial rotates- pectoralis major
Latissimus dorsi, deltoid, teres major,
Subscapularis

82) D

83) D

Muscle attachments of Coracoid process

- The coracobrachialis arises from the medial part of the tip of the coracoid process.
- The pectoralis minor is inserted into the medial border and superior surface of the coracoid process

84) F T F F F

Rotator cuff-

Subscapularis, Supraspinatus
Infraspinatus, teres minor
Subscapularis- Lesser tuberosity
Others-greater tuberosity
Nerve Supply -

- Supraspinatus, infraspinatus- suprascapular nerve (superior trunk)
- Subscapularis- superior & inferior subscapular (Posterior cord)
- Teres minor-axillary nerve (posterior cord)

85) F T T T T

Deltoid (Multipinnate)

- Origin- spine of the scapula acromion lateral 1/3rd of clavicle
- Insertion-deltoid tuberosity of humerus
- Innervation- Axillary nerve (**C5,** C6)
- Action - Major abductor of arm

Clavicular - flexing the arm
medial rotation
Posterior-extending the arm
lateral rotation

86) T F T T T

*Read 79

87) F F F _ T

Clavipectoral fascia-
connects the clavicle to the floor of the axilla
It encloses the subclavius & pectoralis Minor

88) F F T T T

c) Inferior ~~angle~~ – inferior border
d) Musculocutaneous nerve and axillary nerve

Pectoralis major-

- Origin- clavicle, sternum, upper six costal cartilages
  clavicular part passes inferolaterally
- Insertion- lateral lip of intertubercular groove
- Action- adducts arm and rotates it medially, clavicular fibers also flex arm
  sternocostal head extends the arm from a flexed position, particularly against resistance.

**Refer Cunningham pg 32*

89) F F T T T

*Read 82

With the arm above the head, the lowest fibres act with the latissimus dorsi to pull down the arm or raise the body, as in climbing a rope.

90) F T F T F

Serratus anterior –

- Origin-Lateral surfaces of upper 8–9 ribs deep fascia (intercostal spaces)
- Insertion-Costal surface of medial border of scapula
- Action-Protraction, rotation of the scapula
- Nerve supply-Long thoracic nerve (**C5**, C6, C7)

91) T T T F F

Teres major

- Origin-inferior angle of the scapula (Posterior surface)
- Insertion-Medial lip of the intertubercular sulcus
- Innervation-Inferior subscapular nerve (**C5**, **C6**, **C7**)
- Action-Medial rotation and extension of the arm, stabilization joint

92) T _ F F T

Trapezius muscle-

- Origin- Superior nuchal line, external occipital protuberance, ligamentum nuchae, spinous processes of C7-T12 supraspinous ligaments
- Insertion- lateral 3rd clavicle
  acromion
  spine of scapula

- Innervation-

Motor- spinal part of accessory nerve(CN XI). Sensory- anterior rami of C3 and C4

- Actions- Powerful elevator of the scapula,
  rotates the scapula during abduction
  above horizontal
  middle fibers retract scapula
  lower fibers depress scapula

93) T F T F F

c) Latissimus dorsi-

- Origin-Spinous processes of lower six thoracic & lumbar vertebrae, interspinous ligaments, thoracolumbar fascia, iliac crest, lower 3–4 ribs, inferior angle of scapular
- Insertion- Floor of intertubercular sulcus
- Action- Adduction, medial rotation, and
  extension of the arm
- Innervation- Thoracodorsal nerve (C6, **C7**, C8)

d) Long head of triceps- infraglenoid tubercle

94) T T T F T

*Read 86

95) T F F T F

*Read 87

96) T T T F F

Elevation of scapular- trapezius, levator scapulae, rhomboid minor, rhomboid major

97) F F F T T

- Supraspinatus- Laterally rotates arm and stabilizes shoulder joint
- Infraspinatus- Laterally rotates arm and stabilizes shoulder joint
- Subscapularis- Medially rotates arm and stabilizes shoulder joint
- Teres minor- Laterally rotates arm and stabilizes shoulder joint

98) T T T F T

Muscles arising from Interosseous membrane-anterior and posterior compartment of muscles

99) E

Latissimus dorsi is used to pull the arm down from its fully abducted position above head.

Thoracodorsal nerve damage.

(Cunni – Vol1 – pg47)

100) T F F T F

b) Lateral rotator of shoulder joint

c) Medial rotator of scapula

d) Teres minor, sternocostal part of pec. major, latissimus dorsi are adductors of shoulder.

101) F T T T F

a) Pec. minor originates from third to fifth ribs close to costal cartilages.

e) Pec. minor lies anterior to 2nd part of axillary artery.

102) T T F T T

c) Lower 5 digitations of serratus anterior laterally rotates scapula.

103) F T T F T

a) Suprascapular nerve

b) Initiate abduction (15°)

c) Supraspinatus, infraspinatus, teres minor, subscapularis

d) *Refer Cunningham 16th Ed pg81 table6.1*

e) *Refer Ellis 14th Ed pg104*

104) Review FFFFT

F
Upper fibers of trapezius- elevate the scapula
F
Latissimus dorsi- extension, adduction, medial rotation
F
Teres major- adduction, medial rotation
F
Subscapularis- medial rotation
T
anterior fibers of deltoid- flexion, medial rotation.

105) Review FFTTT

F
"SITS" - Supraspinatus, infraspinatus, teres minor, subscapularis
F- They are attached to the greater and lesser tubercles.
T
T
T- The supraspinatus tendon undergoes friction against the acromion during the range of abduction causing pain. This is known as painful arch syndrome. This may result due to supraspinatus tendinitis or subacromial bursitis.

106) TTFFT

a, b, e are originated from lateral epicondyle of the humerus and adjacent intermuscular septum.
c. FDP is originated from anterior and medial surface of the ulnar and anterior medial half of the interosseous membrane
d. FPL is originated from anterior surface of the radius and radial half of the interosseous membrane.

107) Answer _FTTF

a)
b)False- Teres minor originates from the upper 1/3 of the lateral boarder of the scapula.
c)True
d)True- Infraspinatus,teres minor,posterior fibers of deltoid(Lateral rotators)
e)False-Supraspinatus tendinitis leads to painful arch syndrome .It is characterizes by a painful arch of shoulder between 60° and 120°.(ellis page 187)

Muscles of the flexor compartment

108) F T F F F

a) Fused with short head of biceps brachii at origin.

c) Teres minor, sternocostal part of pec. major and latissimus dorsi are adductors of arm.

109) T T F F F

b) Long head of biceps tendon is intraarticular (intracapsular), extrasynovial.

c) Attached to posterior surface of radial tuberosity.

d) Bicipital aponeurosis passes to the posterior surface of ulna.

e) Biceps brachii is a powerful supinator of forearm.

110) T F T F F

b) Inserted into the lateral surface of distal end of the radius.

c) Strong flexor of forearm in mid prone position.

d) Supinator from prone to mid prone position.

e) Supplied by radial nerve.

111) T F T F F

a) Inserted into base of middle phalanges. Flex proximal interphalangeal joint.

d) Flexor digitorum profundus and pronator quadratus are innervated by anterior interosseous branch.

e) Lumbricals originate from flexor digitorum profundus tendon.

112) T T T F T

a) Origin from medial epicondyle of humerus, coronoid process of ulna and anterior border of radius.

d) Inserted into base of middle phalanges. Flex proximal interphalangeal joints.

113) T F F T F

c) Interphalangeal joint of thumb is flexed by flexor pollicis longus. *Read 101st & 102nd reviews.

114) F F F F F

a) Each tendon divides into two slips at proximal phalanx
b) Interosseous muscles arise from metacarpals. Lumbricals arise from flexor digi. profundus tendons.
c) Four tendons of flexor digi. profundus pass through the carpal tunnel and separate in the palm
d) Medial two tendons of flexor digi. profundus is supplied by ulnar nerve while flexor digi. superficialis of same two fingers are supplied by median nerve

115) F F F F T

b) 3rd and 4th goes over 2nd and 5th
c) Flexion of MP and PIP joints
d) Pronator teres has maximum mechanical advantage and initiates pronation

116) T T F F T

117) T F T F –

b) Flexor pollicis brevis and adductor pollicis brevis also flex MP joint

118) F T F T T

c) Pass between flexor carpi ulnaris and flexor digi. profundus
d) Refer Snell 387pg

119) T T F F –

c) Supplied by musculocutaneous nerve
d) Attached to coracoid process

120) Review TFFFF

T- Ulnar artery runs deep to the deep head of the pronator teres while ulnar nerve runs superficial to it.
F- The deep muscle layer of the flexor compartment is supplied by the anterior interosseous branch from the median nerve. Flexor digitorum superficialis is supplied directly by the median nerve.
F- Radial artery lies between the tendons of brachioradialis and flexor carpi radials... Medial to the brachioradialis tendon.
F- Interosseous recurrent artery arises from the posterior interosseous artery.
F- When the nerve is damaged at the wrist, small muscles of the hand are paralyzed. But flexor digitorum profundus is intact. Therefore, flexion of the interphalangeal joints will be more prominent when the nerve is damaged at the wrist. This is called the ulnar paradox.

121) Review FTFTT

F- FDP originates from the Anteromedial surface of the shaft of the ulna.
T- Attached to the distal phalanges of the medial four fingers.
F
T- The medial half of the muscle is innervated by the ulnar nerve and the lateral half is innervated by the median nerve.
T- The distal interphalangeal joints can be flexed only by FDP.

122) TFFTF

b. Deep muscles are not arisen from the medial epicondyle. (FDP, FPL, Pronator quadratus)
c) Medial half of FDP muscle is supplied by ulnar nerve
e) Arises from anterior surface of the radius.

Muscles of the extensor compartment

123) F F F T F

a) Golfer's common flexor origin
Tennis common extensor origin
b) ex. digitorum and ex. indicis

124) F T F T
c) IP joint of thumb remains
d) Refer Cunni. 140pg

125) T T T F F

126) T T F T T
b) Base of 1st metacarpal bone
d) Abduct & extend the thumb

127) T T F T T
c) Middle & distal phalanx
d) *Refer Cunningham 16th Ed pg120*
e) Posterior interosseous nerve

128) T F F T F
a) Lateral – supinator, medial – brachialis
b) Lateral
c) Neck & shaft of radius

129) T T T T F
e) Base of distal phalanx of thumb

130) F F F F F
b) Posterior surface of shaft of ulnar
c) Extend the elbow joint
d) Muscular branch

131) F F T T F
a) Shaft of the ulna
b) Extensor expansion
d) Refer Cunningham 16th Ed pg118

132) T T T T F
Refer Cunningham 16th Ed pg99 fig8.6

133) F – F F T
d) Biceps brachii

134) B

135) Answer FTTFF
a)False-Externsor carpii radialis longus lies in lateral compartment.
b) True-Muscles of the Posterior compartment
Superfical(extensor carpi radialis brevis, extensor digitorum, extensor digiti
minimi,extensor carpi ulnaris, and anconeus.)
Deep(supinator, abductor pollicis longus, extensor pollicis brevis, extensor pollicis longus, Extensor indicis)
c)True
d)False- Lateral compartment of the forearm
e)False-Lateral compartment of the forearm(snell 9th edi,page 392,393)

Muscles of the hand

136) T T F T F
b) Also extend IPJ
e) Adduct

137) F F F T T
a) Median nerve
b) Shaft of metacarpal bone of thumb
c) Median nerve
e) Lateral 2 unipinnate

138) T T F F T
Refer Cunningham 16th Ed pg121

139) F T T F T
Refer Cunningham 16th Ed pg121

140) T T F T T
141) F T F F T

142) T T F T F
c) Palmar interossei
e) Palmar interossei

143) T T F T T

144) – T T T F

e) Palmar interossei function adduction

145) Review FTTFT
F- 4 lumbricals originate from the flexor digitorum profundus tendons.
T- Flex the metacarpophalangeal joints and extend the interphalangeal joints.
T
F
PAD- Palmar interossei adduct the fingers towards the center.
DAB- Dorsal interossei abduct the fingers from the midline.
T

146) Answer FTTF_
a)False-adductor pollicis is supplied ulnar nerve.Other thenar muscles are supplied by reccurent branch of the median nerve.
b)True
c)True-Dorsal interossei abduct the fingers from the line of the middle finger,extend interphalangeal joint,flexion of metacarpophalangeal joint(specialy the 1st dorsal interosseus) (cunningham page 121)
d)False-Lumbricals are located superfical to the mid palmar space.Interossei lies deep to the palmar space.

Muscle testing

147) T T F F T

148) F F – T
a) Foment test is used to test the integrity of deep branch of ulnar nerve
superficial branch supplies palmaris brevis
b) Trapezius is tested by shrugging the chest

149) T T T T F
e) Integrity of ulnar nerve is tested by Foment test

150) T F F F T
Holding a paper between second and third finger is carried out by adduction of fingers. This is done by palmar interossei which is supplied by deep branch of ulnar nerve

Brachial plexus

151) F T F F T

a) Formed by ventral rami

c) Supraclavicular nerves arise from superficial cervical plexus

d) Anterior division of upper and middle trunk forms the lateral cord

e) Cords lie above and lateral to first part of axillary artery

(*Refer Snell 9th edition*)

152) – F – T T

153) T F F T –

b) Axillary nerve innervates deltoid muscle

c) Roots are found in between anterior and middle scalene muscles

154) F F T T T

b) Median nerve is formed by branches of lateral and medial cords

d) Thoracodorsal nerve, upper and lower subscapular nerves arise from posterior cord.

Latissimus dorsi-thoracodorsal

Upper subscapular nerve - subscapularis

Lower subscapular nerve –subscapularis and teres major

155) F T F F F

156) – T T F T

b) Lateral pectoral nerve and long thoracic nerve C_5, C_6, C_7

d) Cords take up their position in the 2nd part of axillary artery

157) F – F F F

158) F – F F F

159) F F T T F

160) - F – F F

161) C

162) T T T F T

163) T T F T T

Damage to C5 to C6 roots cause Erbs-duchenne paralysis

c) Elbow extension is done by triceps. Triceps is not paralyzed in Erbs-duchenne paralysis. *(Refer Ellis 14th edition Pg215)*

164) A

165) T T T T F

c) Median nerve is formed by both medial and lateral cords

e) Long thoracic nerve is formed by C5, C6, C7 roots.

166) T T T F T

a) Median nerve, pronator teres and quadratus is paralysed.
b) Median nerve, opponens pollicis is paralysed.
c) Median nerve, flexor digitorum superficialis is paralysed.
d) Radial nerve
e) Median nerve, FCR, PL, FDS, FPL & part of FDP are paralysed.

167) Review FFTTF

F- C5-T1 ventral rami
F
Roots- Between scalenus anterior and medius muscles
Trunks- In the posterior triangle of the neck
Divisions- Behind the middle 1/3 of the clavicle
Cords- In the axilla
T
T
From the roots- Nerve to subclavius, dorsal scapular nerve, long thoracic nerve
From the upper trunk- Suprascapular nerve
F- Ulnar nerve is from the medial cord

Nerves of the Upper limb

168) F T T F F

a) Only when arm is abducted
d) Lateral
e) Impaired, brachialis & biceps are paralysed which are innervated by musculocutaneous nerve

169) T F F F F

b) Above
c) Median N supplies thenar muscles & lateral 2 lumbricals
d) Musculocutaneous N
e) Above

170) B

Opponens pollicis - median nerve
Opponens digiti minimi - ulnar nerve

171) T F T T T

a) Against the clavicle
b) Medial
c) Deep branch of radial nerve becomes posterior interosseous nerve after piercing supinator
d) At mid arm level

172) T F F F T

a) Policeman's badge sign
b) ECRL intact, supplied from the radial nerve above its division; it alone is sufficiently powerful to maintain extension of the wrist.
c) Klumpke's palsy
d) Ulnar paradox
e) Due to loss of opposition

173) T T F - -

a) *Cunningham 16th edition pg144*
b) *Cunningham 16th edition pg 82& 129*
c) Median nerve

174) T - T F F

c) Upper posteromedial part of the arm *Cunningham 16th edition pg 63*
d) Musculocutaneous N
e) Wrist drop

175) F F F F F

b) Ventral rami
c) Musculocutaneous N
d) From upper trunk of brachial plexus

176) T T T T T

a) *Cunningham 16th edition pg 116*

c) Lateral cutaneous nerve of the forearm

e) *Cunningham 16th edition pg 141*

177) F T F F T

a) Lower triangular space

c) Lower part of the deltoid

d) C5

178) T T T T T

a) Ulnar paradox

b) Ulnar nerve supplies all the muscles of the hand except thenar muscles & lateral 2 lumbricals

d) Deltoid & supraspinatus are paralysed

179) B

Intercostobrachial nerve supplies the skin of the medial side of the arm and the floor of the axilla.
Cunningham 16th edition pg 29

180) C

Klumpke's palsy, T1 nerve root is affected. Palmar interossei are wasted, unable to keep the paper, cutaneous supply to medical side of forearm is by T1.

181) F T F T T

a) Supraspinatus - suprascapular nerve

c) Pec minor- medial pectoral nerve

182) TTFFF

b. C6 - radial side of forearm & thumb

c. C7 - skin of the hand except thumb & little finger

d, e. C8 - Ulnar side of forearm & little finger

183) Answer B

Here, the suprascapular nerve, nerve to subclavius, musculocutaneous nerve, and axillary nerve, which derived from the C5 and C6 roots, are functionless.

a. The floor of the axilla is supplied by the intercostobrachial nerve (lateral cutaneous branch of the second intercostal nerve).

b. Supplied by the axillary nerve.

c. Deltoid, latissimus dorsi, and teres major take part in this action. Although the deltoid is paralyzed, weak extension is brought about by latissimus dorsi and teres major.

d. Triceps are responsible for elbow extension mainly (supplied by the radial nerve).

e. The muscles of the flexor compartment are not affected.

184) Answer C

a. Thumb abduction is weakened not impaired. Abductor pollicis longus is paralyzed in injury to the radial nerve in elbow region. But

weak abduction is brought by abductor pollicis brevis.

b. Medial side of dorsum of hand is supplied by dorsal branch of ulnar nerve. Here ulnar nerve is not affected

c. weak extension of interphalangeal joint is brought by interossei and lumbricals

d. Posterior aspect of forearm is supplied by posterior cutaneous nerve of forearm, branch of radial nerve which is given in the arm.

e. Nerve supply to the supinator and the extensor carpi radialis longus will be undamaged and these muscles are powerful to keep the wrist joint extend and wrist drop will not occur.

185) Answer A

The intercostobrachial nerve is the lateral cutaneous branch of the 2nd intercostal nerve. It supplies the skin of the armpit and the upper medial side of the arm. Since it is a content of the axilla, it could be damaged during the procedure of removing axillary lymph nodes. The medial cutaneous nerve of the arm also supplies the upper medial side of the arm, but it does not supply the axilla. Additionally, it usually does not get damaged during a mastectomy.

186) Answer _TTTT

a)
b)True-dorsal surface of the thumb is supplied by radial nerve.
c)True-It has an cutaneous supply because lateral
cutaneous nerve of the forearm is given by
musculocutaneous nerve.
d)True
e)True-Ulnar claw hand occurs due to paralysis of
interosseus muscles and medial 2 lumbricals

Axillary Nerve

187) F F F F T

a) C5, C6
b) Only deltoid & teres minor
c) Upper lateral cutaneous nerve of the arm
d) During surgical neck fractures

188) F F T T F

a) Upper lateral cutaneous nerve of the arm
b) Teres major - lower subscapular nerve
d) *Cunningham 16th edition pg 74*

189) T T T T F

a) Deltoid
b) Posterior fibres of deltoid
c) Clavicular part of deltoid
d) Posterior fibres of deltoid & teres minor
e) Pec major, teres major, latissimus dorsi

190) T T F T T

a) 15°to 90° abduction by deltoid which is supplied by Axillary nerve.
c) Anterior division of axillary nerve accompany the posterior circumflex humeral vessels.

Musculocutaneous nerve

191) F T F T T

a) Arises from lateral cord.
c) Innervate coracobrachialis, biceps, brachialis.

192) F T T T F

a) Arises from lateral cord.
e) End up as lateral cutaneous nerve of forearm.

Median nerve

193) T F T F T
b) Medial root of median nerve - C8, T1
d) Lie between two heads of pronator teres.
e) Lie between flexor carpi ulnaris and palmaris longus at wrist.

194) F T F F F
b) Thenar eminence muscles and lateral two lumbricals are supplied by median nerve.
c) All palmer and dorsal interossei are supplied by ulnar nerve.
e) Medial to the biceps tendon at cubital fossa.

195) T T T F F
e) Ulna nerve supplies all interossei muscles of hand.

196) A
b) Supinator - posterior interosseous nerve
c) Flexor carpi ulnaris- ulnar nerve
d) brachioradialis- radial nerve
e) FDP - median nerve and ulnar nerve

197) F T F T T
a) Supinator -deep branch of radial nerve, biceps - musculocutaneous nerve.
b) Pronator teres, pronator quadratus
e) Flexor pollicis longus

198) F T T F F
a) In supine position.
c) Due to paralysis of flexor pollicis longus.
d) Median nerve give sensory supply to palmer surface of lateral 3 ½ digits and distal end of dorsal surface of lateral 3 ½ digits.

199) F F T T T
a) Sensation of back of index finger is supplied by radial nerve except most distal end which is supplied by median nerve.
b) Adductor pollicis is supplied by ulnar nerve.
c) due to paralysis of opponens pollicis.
d) Lateral two lumbricals are supplied by median nerve.
e) Due to paralysis of radial side of FDP and FDS.

200) T F T T F
a) Sensation of most distal end of dorsum of lateral 3 ½ digits are supplied by median nerve.
b) Abduction of thumb occur by abductor pollicis brevis which is supplied by median nerve and abductor pollicis longus which is supplied by radial nerve. Therefore abduction of thumb is weak.
e) Cannot flex.

201) T T F T F
b) Due to paralysis of flexor pollicis longus.
c) It has no sensory branches.
d) Medial side of FDP is supplied by ulnar nerve.

202) T F - T T
b) FDP to middle and index fingers.

203) FFTTT
a. by deep branch of radial nerve

b. by deep branch of radial nerve

c. by Anterior interosseous branch of median nerve

d. by median nerve

e. by median nerve

Ulnar nerve

204) F T T F F
a) Radial nerve
d) Lateral cutaneous nerve of forearm supplies the anterolateral surface of forearm.
e) More deformity at wrist due to ulnar paradox.

205) T T F T T
d) Deep branch supply to hypothenar muscles, medial two lumbricals, dorsal and palmer interossei and adductor policis.

206) T F F T T
b) Thenar eminence muscles are supplied by median nerve.
d) *Refer Snell 9th edition 392 page image*

207) F F - - F
a) Superficial
b) C8, T1
e) Ulnar paradox

208) F F F F F
a) Can abduct. Abductor pollicis brevis is supplied by median nerve.
c) Medial surface of forearm is supplied by medial cutaneous nerve of forearm.
d) Part of FDP

209) F T T T T
a) On flexor retinaculum *(Cunni-Vol 1-pg 107)*
c) Diagram *(Snell pg 446)*

210) T F F F T
b) Extensors supplied by radial nerve can act
c) Adduction is impossible

211) F T T F F
a) **Refer Snell pg 492*
c) Ulnar paradox
e) **Refer Snell pg 492*

212) FTFFT
a. Medial 2 lumbricals are paralyzed in ulnar nerve damage. Lateral 2 are paralyzed in median nerve damage.
c. Ulnar paradox occurs when ulnar nerve damage occurs at the wrist joint.
d. Occurs due to Median nerve damage

Radial nerve

213) T T F T F

214) T F T F F
b) Extensor carpi radialis longus intact *(Ellis pg 118)*
e) Pierce at lower 1/3 humerus divide at lateral epicondyle

215) T F F F F
d) **Read 190 b) review*
e) Biceps intact *(Snell pg 489)*

216) T T F T F
c) deep to brachioradialis *(Ellis pg 113)*

217) F F F T F
d)**Refer Snell pg 390*

218) F T F T T
c) By lateral cutaneous nerve of forearm

219) T T F F T
a) Because wrist drop
b) Biceps is the only supinator. It is already active in flexion.
c) Small loss due to overlap
d) By lumbricals & interossei *(Snell pg 489)*

220) T T F F F
e) Abductor pollicis brevis intact

221) T T F F F
a) **Refer Cunningham Vol 1-pg 118)*
d) To abductor pollicis longus

222) B

The main trunk of radial nerve supplies the triceps, anconeus, brachioradialis, extensor carpi radialis
longus & lateral part of brachialis before the bifurcation at lateral epicondyle. Brachioradialis is supplied most distally.

223) F T F T F
a) Lateral epicondyle
c) Neck of radius

224) Review TFFF_
T- Extensor Carpi radialis longus, extensor carpi radialis brevis, and brachioradialis muscles (extensors of the wrist) are innervated by the main trunk of the radial nerve in the mid-arm. Damage to the main trunk can cause wrist drop as the powerful extensors like extensor carpi radialis longus are not intact now.
F- Extension of the elbow is not lost, because the triceps muscle is intact.
F- Elbow flexors are not innervated by the radial nerve.
F- Only the sensation over the first web space is lost.
_ Supination is not lost because the biceps is intact. But the action becomes weak as the action of the supinator is lost.

Upper limb vessels & nerves

225) TT T F F
a) In anatomical snuff box
b) Deep - level of proximal border of extended thumb
Superficial - level of distal border of extended thumb
c)**Refer Ellis pg 98*
d) Overlying
e) Posterior

226) F F F T T
**Refer Cunningham pg 88*

227) F F T T F
a) Level of neck of radius
c) **Refer Ellis pg 97*
e) Deep arch *(Cunni- Vol 1- pg 112)*

228) T T F T
a) **Refer Snell pg 426*
b) **Refer Cunningham pg 89*
d) **Refer Cunningham pg 106*

229) F F T F T
a) Posterior circumflex humeral artery accompanies the axillary nerve
b) Lateral thoracic artery accompanies the long thoracic nerve

230) F T F T F
a) Leave the neurovascular bundle piercing the coracobrachialis muscles
c) Not a content of the neurovascular bundle
e) The 1st nerve to leave the neurovascular bundle is the radial nerve. Occurs before the coracobrachialis insertion.

231) F T T T T
a) The neurovascular bundle of arm lies anteriorly and parallel to the medial intermuscular septum.

232) Review TTTTF
T- The ulnar nerve is accompanied by the superior ulnar collateral artery (branch of the brachial artery). It pierces the medial intermuscular septum halfway down the upper arm.
T- Branches to the medial and long head of the triceps and posterior cutaneous nerve of the arm are given in the axilla.
T
T- Cutaneous supply to the lower part of the deltoid is from the axillary nerve.
F- The nutrient artery to the humerus is directly from the brachial artery.

Arteries of the Upper limb

233) F T T T F
a) Begins at the lateral border of the first rib.
e) Mainly artery lies medially to the arm muscles.

234) F T T T T
a) Axillary vein lies lateral to the axillary vein.

235) F T F T
a) Radial artery enters the palm between 1st & 2nd metacarpal bones and the two heads of the 1st dorsal interosseous muscle. Then it curves medially between two heads of adductor pollicis muscle.
c) Radial artery accompanies the superficial branch of the radial nerve.

236) F F T F T
a) Radial artery is the artery of the hand and ulnar artery is the artery of the forearm.
b) At the level of neck of radius
d) Become superficial palmer arch after crossing flexor retinaculum.

237) F F F F F
b) Main contributor of deep palmer arch is radial artery.
c) Has a contribution
d) Give rise to the proper digital artery of digit 5.

238) T T T F T
d) Do not pierce. Cross over the upper border of the interosseous membrane.

239) T T T T T

240) F T F F F
a) Lateral border of the 1st rib
c) Ulnar artery is larger than the radial artery.
d) Passes between two heads of the adductor pollicis.
e) Superficially

241) T T F T F
c), e) Deeply placed

242) F T T F T
a) Below the elbow joint
b) It anastomoses with the anterior interosseous artery at distal end of forearm. Joins the dorsal carpal arch.

243) T T T F T
d) Ischemia can occur at distal part of middle finger.

244) T F F F T
b) Along the medial border of scapula
c) Not limited due to scapular anastomosis
d) Scapular anastomosis- Dorsal scapular artery, suprascapular artery, circumflex scapular artery, posterior circumflex humeral artery.

245) F T T T F
a) Radial artery is the artery of hand.
d) Anastomosis in elbow
e) At the neck of the radius

246) T F T T T
b) Posterior interosseous artery medially related to the deep branch of the radial nerve.

247) T T F F T
c) Superficially
d) Deep palmar arch is formed by radial nerve, but has a little contribution from the ulnar artery.

248) T - T T F
b) The interosseous arteries are distributed to the muscles lying in front of and behind the interosseous membrane; they provide nutrient arteries to the radius and ulna bone. (*Snell Page 388)*

249) F F F T T
a) Neck of the radius
b) Given by ulnar artery
c) Accompanies the superficial branch of radial nerve

250) TTFTT

a. Ulnar artery passes deep to the muscles arising from the medial epicondyle (common flexor origin)

b. It passes deep to the two heads of pronator teres

c. Ulnar artery lies lateral to ulnar nerve at distal 2/3 of forearm

d. UA cross the flexor retinaculum to form the superficial palmar arch with the superficial branch of the radial artery

e. Superficial palmar arch supply the medial 2/3 fingers by four digital branches.

251) Answer FFTTT

a)False- Subclavian artey becomes axillary artery after it passes the outer border of the 1st rib
b)False-contents of the axillary sheath Axillary artery ,brachial plexus.Axillary vein is not a content.(kuppi note page 24)
c)True -axillary artery is divide into 3 parts by pectoralis minor muscle.2nd part of the axillary artery is covered by pectoralis minor.Thoracoacromial artery arise from 2nd part.Therefor it lies deep to the axillary artery.
d)True-Subscapular artery is the largest branch of the axillary artery.
e)True-In the 1st part of the axillary artery all cords lie posterior to the arteryIn the 2nd part of the axillary artery cords take up the positions as they indicated by their names.

Scapular anastomosis

252) F - F F -

d) Branch of subscapular artery

253) T T T T F

254) F T T F T

255) Review TTTFT

Branches from subclavian artery- suprascapular artery, superficial cervical artery which gives off a deep branch

Branches from axillary artery- subscapular artery (this artery divides into circumflex scapular artery and thoracodorsal artery), anterior and posterior circumflex humeral arteries.

256) Answer TTTFF

a)True-Arteries that contributes to scapular anastomosis Suprascapular artery,Deep branch of the transverse cervical artery,circumflex scapular artery,Thoracodosal artery)
b)True
c)True
d)False
e)False

Veins of the Upper limb

257) T T F T T
c) The basilic vein ascends in the superficial fascia on the medial side of the biceps (Fig. 9.39). Halfway up the arm, it pierces the deep fascia and at the lower border of the teres major joins the venae commitantes of the brachial artery to form the axillary vein. (*Snell Page 369)*

258) F F F F T
a) Lateral end
c) The cephalic vein ascends in the superficial fascia on the lateral side of the biceps and, on reaching the infraclavicular fossa, drains into the axillary vein. (*Snell Page 369)*

259) F T T F T
a) The largest superficial vein in the body is the Great Saphenous Vein.

260) T T T T T
d) In the cubital fossa, the median cubital vein is separated from the underlying brachial artery by the bicipital aponeurosis. This is important because it protects the artery from the mistaken introduction into its lumen of irritating drugs that should have been injected into the vein. (*Snell Page 370)*

261) F T F T T
a) Runs upwards and medially to join the basilic vein.

262) T T T T T
b) FIGURE 9.41 (*Snell Page 371)*

263) F T F T F
e) Posterior to radial styloid process

264) T F T F F
d) Drains into the axillary vein

Axilla

265) F T F F T
c) Cords of brachial plexus

266) F F F F F

267) F T T T T
a) Anterior axillary fold is formed by the pectoralis major. *(Cunningham – 16th edition pg.28)*
b) (Cunningham – 16th edition pg.28)

268) T F T T F
a) Medial wall is formed by lateral aspects of upper 4 intercostal spaces, digitations of serratus anterior. (*Ellis 13th edition -pg.197*)
b) Structures that pierce the clavipectoral fascia,
C – Cephalic vein
A – Acromiothoracic artery
L – Lateral pectoral nerve
L – Lymphatics

269) F T T T T

270) F F T F F
a) Long thoracic nerve descends in the lateral surface of serratus anterior (*Cunningham 16th edition – pg.37*)
b) Medial wall
d) Anterior
e) Axillary sheath contains three cords of brachial plexus and the axillary artery.

271) F T F F T
a) Medial border
c) Long thoracic nerve
d) Cords of brachial plexus

272) F T T F T
a) Medial margin of apex of axilla
d) Axillary sheath encloses cords of brachial plexus and axillary artery
e) Crutch palsy *(Ellis 13th edition- pg.215)*

273) T T T F
b) Apex is anteriorly bounded by the middle third of clavicle. *(Ellis 13th edition – pg.197)*
d) Posterior wall is formed by subscapularis, teres major, latissimus dorsi**.** *(Ellis 13th edition – pg.197)*

274) F T T _ T
Refer *Ellis 13th edition pg.211*

275) F F T T F
a) Pectoral / anterior group lying deep to pectoralis major along the lower border of pectoralis minor *(Ellis 13th edition- pg.212)*
b) Can be palpated
d) *Ellis 13th edition pg.212*
e) Along the subscapular vessels *(Ellis 13th edition- pg.212).* Receive lymph from posterior wall of upper half of trunk and from the **axillary tail of breast.**

276) T T F F T
a) *(Ellis 13th edition – pg.212)*
b) Lying deep to pectoralis major along the lower border of pectoralis minor. *(Ellis 13th edition- pg.212)*
c) Superficial lymph nodes initially drains into the cubital lymph nodes and then terminate in to the axillary lymph nodes. Deep lymphatic vessels terminates in to humeral axillary lymph nodes. Finally all the axillary lymph nodes drain in to apical lymph nodes.
d. along the axillary vein *(Ellis 13th edition- pg.213)*
e) *(Ellis 13th edition- pg.214)*

277) T T F F T
a) Central lymph nodes drains lymph from lateral, anterior and posterior lymph node groups. Posterior lymph nodes receives lymph from skin and the muscles of the posterior and inferior body wall.
c) Lateral nodes locates along the axillary vein. Central nodes located in the axillary fat.

d) Along the subscapularis vessels *(Ellis 13th edition- pg.212)* in the inferior border of the subscapularis muscle.

278) Review FTTTF

F- lateral wall is formed by the medial aspect of the upper arm (humerus, biceps brachii, coracobrachialis)
T- Long thoracic nerve runs down on the medial wall of the axilla.
T
Anterior group- Along the lower border of pectoralis minor, deep to pectoralis major
Posterior group- Along the subscapular vessels
Lateral group- Along the axillary vein
Central group- In the axillary fat Apical group- Immediately behind the clavicle at the apex of the axilla above pectoralis minor and along the medial side of the axillary vein. (Ellis)
T- Subclavian artery --> Axillary artery--> Brachial artery
F- The largest branch is the subscapular artery.

Cubital fossa

279) F F T T F

a) Triangular intermuscular space. *(Ellis 13th edition- pg.197)*
b) Formed by the brachialis muscle and capsule of elbow joint behind it. *(Ellis 13th edition- pg.198)*
e) Is a content of the roof of the cubital fossa.*(Grant's Atlas 13th edition – pg.546)*

280) F F T T T

a) Nerve to ECRL is given from the Radial nerve before the Cubital fossa.
b) Separated from the bicipital aponeurosis.
e) *Grant's Atlas of Anatomy 13th edi. pg.548*

281) F T F T

a) Upper part of the floor is formed by the brachialis and the lower part is formed by the Supinator.
c) Contents from medial to Lateral are, The median nerve, the brachial artery , tendon of biceps brachii. If the brachioradialis muscle is undermined and retracted laterally, the radial nerve can be brought into view. *(Ellis 13th edition- pg.199)*

282) F F F T F

a) Medial border is formed by the lateral margin of pronator teres.
b) Roof is formed by the overlying deep fascia, which is **partially reinforced by the bicipital aponeurosis**. *(Ellis 13th edition- pg.198)*
c) Epitrochlear lymph nodes / Cubital lymph nodes / Supraepitrochlear lymph nodes

283) A *(Ellis 13th edition- pg.176, 174)*

284) T T T F T

b) In the cubital fossa brachial artery divides in to ulnar and radial arteries. The radial artery leaves the fossa at the apex and the ulnar artery leaves by passing deep to the pronator teres. *(Cunningham 16th edition – pg.90)*

c) *(Cunningham 16th edition – pg.90)*

d) At the middle of the arm ulnar nerve pierce the medial intramuscular septum and enters to the posterior compartment of forearm. *(Grant's Atlas 13th edition – pg.548)*

e) Is a content of the roof of fossa *(Ellis 13th edition – pg.198)*

285) T T F T T

c) It passes deep to the median cubital vein and superficial to median nerve and brachial artery. *(Grant's atlas 13th edition – pg.547)*

d) *(Grant's atlas 13th edition – pg.547)*

286) F T T F T

a) Medial border is formed by lateral border of pronator teres

c) Contents of roof: Skin, superficial fascia, median cubital vein, lateral cutaneous nerve of forearm, medial cutaneous nerve of forearm, deep fascia reinforced by bicipital aponeurosis.

d) Floor is formed by brachialis and supinator muscles.

287) F T T F T

a) Lateral border of pronator teres

b) Brachialis and Supinator

c) *(Cunningham 16th edition – pg.90)*

288) FTFFF

a. lateral border of pronator teres

c. bicipital aponeurosis forms the roof of cubital fossa

d. median cubital vein is in the roof
Contents - radial nerve, tendon of biceps, brachial artery, median nerve

e. medial nerve is the most medial content, brachial artery just lateral to it

289) Answer

a)True-The floor of the cubital fossa is formed by brachalis and supinator muscels.
Medial border(Lateral border of pronator teres)
Lateral border(Medial border of brachioradialis)
Base(Imaginary line drawn between 2 epicondyles)

b)True(refer ellis page 198)

c)False- lateral border of pronator teres

d)False

e)True-Deep head of pronator teres separates the ulnar artery and median nerve.Median nerve lies above the head and ulnar artery lies deep to the head(snell 9th edi,page 390)

Carpal Tunnel

290) T T T F F

a) *(Snell 9th edition – pg.384)*

b) Originates from tubercle of trapezium and flexor retinaculum.

d) Tendon of FCR is within the carpel tunnel in a separate compartment. *(Snell 9th edition – pg.384)*

291) T T F F F

b) *(Snell 9th edition – pg.384)*

c) Ulnar nerve and artery passes superficial to the carpal tunnel. *(Snell 9th edi – pg.384)*

d) Radial artery passes posterior to the carpal tunnel. *(Snell 9th edition – pg.384)*

e) Anterolaterally. *(Snell 9th edi – pg.384)*

292) F F F T F

c) The tendon passes deep to the flexor retinaculum in a separate osseofibrous canal, produced by the groove on the trapezium and the flexor retinaculum. So that the tendon of flexor carpi radialis is located outside the carpal tunnel *(Kulkarni pg 140)*

Hand

293) T F F T T

a) The medial part of the palmar aponeurosis undergoes progressive shortening in this condition. *(Kulkarni pg 161)*

b) Ulnar N.

c) Palmar Aponeurosis lies anteriorly to both superficial and deep palmar arches.

d**)** P-AD D-AB Dorsal Interossei abduct the fingers.

e) The continuation of the radial artery in the palm mainly forms the deep palmar arch, which is completed on the medial side by the deep branch of ulnar artery. *(Kulkarni pg 165)*

294) T T T T T

a) *Cunningham vol 1 pg 65*

b) *Cunningham vol 1 pg 65*

c) The muscles of the thenar eminence are paralyzed and wasted so that the eminence becomes flattened. The thumb is laterally rotated and adducted. The hand looks flattened and "apelike." *(Snell pg 432)*

d) The long flexor muscles of the carpus and fingers are more contracted than the extensor muscles, and the wrist joint is flexed; the fingers are extended. *(Snell pg 383)*

e) Results from fibrous contraction of the palmar aponeurosis, particularly of the 4th and 5th fingers.

295) T T T F F

a) At its apex

b) (*Cunningham pg 66 – Deep Fascia of the Upper Limb.)*

c) (*Ellis pg 211 – Spaces of the hand)*

e) Dupuytren's Contracture

296) – F F F F

a) The synovial sheaths of the 2nd, 3rd and 4th digits are independent and terminate proximally at the levels of the heads of the metacarpals. The synovial sheath of the little finger is continuous proximally with the ulnar bursa, and that of the thumb with the radial bursa.*(Chaurasia pg 130)*

b) It's Radial Bursa

c) Midpalmar space lies behind the flexor tendons of medial 3 digits covered with Ulnar Bursa.

e) Does not communicate with common synovial sheath.

297) T F T T T

a) (*Cunningham pg 137 – Fibrous capsules of Metacarpophalangeal joints)*

298) T F T T

a) At the metacarpophalangeal and interphalangeal joints, the extensor expansion forms the dorsal part of the fibrous capsule of the joint. It is continuous laterally with the remainder of the fibrous capsule and is held in position by it. *(Cunningham pg 121)*

b) Extensor expansion is Triangular in shape.

299) – T F T T
c) Lumbricals, interossei also flex M-P joints.
d) The proper palmar digital arteries to each finger form a rich anastomosis in the pulp of the finger and in the nail bed. *(Cunningham pg 102)*
Palm and Palmar surfaces of digits has a High density of nerve endings.
e) Lumbricals are innervated by Median and Ulnar Nerves.

Anatomical snuff box

300) F F T F T
a) Medially
b) Extended
d) Cephalic vein lies in the roof. The only structure that passes through it is the radial Artery.

301) T F T T F

302) C

303) T F T T T
c) Trapezium*

Flexor retinaculum

304) F T T F F

305) T T T F T

306) T F F T T
c) Tubercle of the Scaphoid, Pisiform, Tubercle of the Trapezium, and hook of the Hamate are all palpable.
d) Pocessed*

307) F T F T T

308) F T F F F

309) F F T F T

310) T T T T F

Spaces of the hand

311) T F T T T
b) 2nd and 3rd Metacarpals and adductor pollicis.
c) Both radial and ulnar bursa but the thenar and mid palmar spaces extend proximal to the Flexor Retinaculum.
d) *Refer Ellis pg.120*

312) T F T F F
c) Space of Parona boundaries: Anteriorly- Flexor digitorum profundus/ Posteriorly-Pronator quadratus
d) Approximately in 50% cases they communicate.
e) Thenar space.

313) T _ F T T
a) Closed fascial compartments located in front of the terminal phalanx pf each finger subdivided by small fibrous septae.
c) Diaphysis (epiphysis supplied from branches of digital arteries more proximally)

314) T T T F T
c) Digital synovial sheaths of 1st and 5th fingers are continuous with radial and ulnar bursae respectively where that of 2nd, 3rd and 4th are closed (attached to the periosteum) at metacarpophalangeal joints.

d) Loose attachments of skin of the dorsum causes a site for fluid collection. *Read Ellis pg. 120.*

315) F T _ T _

b) Compression of blood vessels in vincula longa and brevia

316) F F T F T

c) Web spaces communicate with one of palmar spaces via lumbrical canal.

d) Located behind the thenar space. Not a content, it is a posterior boundary.

317) T F F T T

a) 1st web space with thenar space

b) The digital synovial sheaths can involve due to the close relation of the proximal end of pulp space with distal end of synovial sheath.

c) Proximally thenar and mid palmar spaces are closed off from the forearm by the walls of carpal tunnel.

318) T F T F T

b) By adductor pollicis

c) Potential spaces filled with loose connective tissues.

d) By oblique fibrous septum

319) F T T T F

b) *Refer Ellis 213pg*

d) Pus collection may spread to the dorsum due to the loose attachment of skin with underlying structures.

320) C

Compression of blood vessels causes necrosis and it causes pain.

321) T F T T T

b) 1st lumbrical and long flexor tendon of index finger.

322) A

Joints of the Upper limb

323) F T T T F

a) Hyaline

e) Both ellipsoid and condylar joints have similar features but they are two types.

- Condyloid: 2 distinct convex surfaces articulate with two concave surfaces. Biaxial joints allows flexion, extension, abduction, adduction and small amount of rotation. Ex: Metacarpophalangeal joints.
- Ellipsoid: Elliptical convex articular surfaces fitting into an elliptical concave articular surface. Flexion, extension, abduction adduction are possible where rotation is absent.

324) T F T T –

b) Inferiorly

325) F F T F T

a) Subacromial bursa

b) Tendon of long head of Triceps

d) Damage to ulnar nerve causing paralysis of intrinsic muscles and Flexor digitorum profundus

e) Separated by the triangular articular disc from distal border of Radius to the base of ulnar styloid process.

326) T F F T T

b) Hair line fractures, no separation of bones and heals rapidly. Commonly seen in children.

c) First turns to cartilage and then ossifies.

327) T F F T F

b) 1st carpometacarpal joint

c) Synovial saddle shaped

e) Synovial hinge type.

Shoulder joint

328) T T T T T

e) As soon as abduction

commences at shoulder joint (90^0), rotation of scapula begins.

329) T T T F F
d) Joint capsule extend down on to the diaphysis on the medial aspect of neck of humerus.

330) T F T T T
b) Not always

331) F _ T T T

332) T T T F T
d) Normally dislocate anterior inferiorly.

333) T F T F F
d) Anatomical neck

334) _ _ T _ _
c) Produced by teres minor, deltoid, infraspinatus

335) _ T T F _
c) Biceps brachi

336) T F T T F
b) subscapular bursa
c) Rotator cuff muscles ,long head of biceps brachi & more distantly related long muscles of shoulder - deltoid, long head of triceps, pectoralis major, latissimus dorsi, teres major

337) E
Joint's inferior aspect is completely unprotected by muscles and joint capsule is weak in in its inferior part.

338) T _ F T _
c) long head of bicep

339) A

340) TFTFF

b. long head of biceps lies within the capsule.
d. 1° to 15° is done by Supraspinatus, 15° to 90° is done by Deltoid, beyond 90° is done by action of Serratus anterior and Trapezius.

341) Answer TTFTT
a)True
b)True- The capsule attached arround the epiphyseal lines Of the both glenoid and the humeral head and it also extends to the diaphysis on the meadial aspect of the neck of the humerus.Therefore osteomyelitis of upper end of humeral shaft may invole the joint.(ellis page 185)
c) False
d)True- Latissimus dorsi,teres major,triceps brachii,posterior fibers of deltoid(Extensors of the shoulder joint)
e)True-The tendon of the long head of the bisceps brachii is extra syanovial and intra capsular.

Sternoclavicular joint

342) T F T F T
d) Sternoclavicular joint is a double plane joint. Articular disc lies within cavity & divide joint's interior in to two.

343) F T _ F _
a) Synovial double plane joint
d) Synovial joint

344) T T T F F
e) Posteriorly on the right - brachiocephalic artery, on the left - left brachiocephalic vein & left common carotid artery.

345) T T _ T _

346) Answer TTTFT

a)True-sternoclavicular joint is a synovial duoble plane joint
b)True-Posteriorly important relations
Right:brachiocephalic artey
Left:left common carotid artey,left brachiocephalic
artery(snell 9th edi,page 362)
c)True- The main stability factor of the sternoclavicular joint is the ligaments,particularly the sternoclavicular and costoclavicular ligaments.
d)False
c)True-Arch of the aorta is positioned posteriorly to the
manubrium of the sternum.The sternoclavicular joint located at the junction of the clavicle and the sternum anterior to the aortic arch.

Elbow Joint

347) T T T T T

348) T T T T T

349) T T T T F

350) E

351) T F F T T

b) Articular surfaces – wrench shaped articular surface of olecranon & pulley shaped trochlea
c) Biceps brachii

352) Review TTTFT

T
Shoulder joint- synovial incomplete ball and socket joint
Elbow joint- synovial hinge joint
Proximal and distal radioulnar joints- synovial pivot joint
Wrist joint- synovial condylar joint
T- Trochlea and the capitulum of the humerus articulate with the trochlear notch of the ulnar and head of the radius respectively.
T
F
T- Olecranon bursitis leads to swelling of the tip of the elbow, causing student's elbow.

Radio-ulnar joint

353) F T F T

a) Synovial pivot joint

354) T T T F

d) Supination is more powerful movement because of the strength of the biceps muscle.

355) F F F T T

a) Pronation – pronator teres, pronator qudratus, & *read 317*
b) Axis passes through the head of the radius above and the attachment of the apex of the triangular articular disc below.

356) T F F T

c) Pass proximal to the interosseous membrane, anterior interosseous vessel pierce the membrane

357) Review TTFT_

T- Proximal and distal radioulnar joints are synovial pivot joints.
T- The annular ligament forms a collar around the head of the radius.
F- The proximal radioulnar joint communicates with the elbow joint while the distal radioulnar joint doesn't communicate with the wrist joint.
T- Pronator teres is innervated by the median nerve.

Wrist joint

358) T F T T F

c) Posterior dislocation common in carpal bones of wrist except lunate
anterior dislocation common in lunate
e) Adduction > abduction – because greater distal projection of the radial styloid
Extension > flexion
Rotation is not possible

359) F T T T F
a) *Cunningham's volume 1 page 130*

360) F F T T T

361) F F F F F
b) To scaphoid, lunate & triquetrum
d) Abduction
e) To scaphoid

362) F T F F F
363) T F F F F
c) Only scaphoid & lateral part of lunate

364) T T F T F
c) Scaphoid, lunate, triquetrum
e) C_7 , C_8

365) Answer TFFTF
a)True
b)False-wrist joint does not communicate with the distal radio ulnar joint because those are separated by the articular disc of distal radio ulnar joint.
(cunningham page 133)
c)False-Ulnar dose not participate for the formation of wrist joint because articular disc sperates the
ulnar from the joint.(cunningham page 130)
d)True
e)False-The range of abduction is less than adduction because of greater projection of the styloid process of the radius.

Metacarpophalangeal joint

366) F F T T T
a) Synovial condyloid joint
b) Firmly attach to phalanx, less to the metacarpal

367) F T T T

Upper Limb Deformities

368) T T F F T
c) *Refer Snell page 490*
d) Flexor digitorum profundus & flexor pollicis longus

369) T T T T F
Refer Snell page 488 – 492

370) T F F T T

b) *Refer Gray's anatomy - 4th edition – page 686*

c) Damage to long thoracic nerve

371) B

372) T T F T F

c) Abductor pollicis muscle is innervated by deep branch of ulna nerve

e) Less sensation

373) F T T T F

Refer Ellis – 13th edition – page 215

374) B

375) F T F T F

c) Due to paralysis of teres minor and supraspinatus

e) Opponens pollicis muscle is innervated by C8 and T1

376) C

Due to damage to the palmar cutaneous branch of medial nerve

377) A

378) D

Wrist extension does not less due to action of extensor carpi radialis longus

379) T T T T T

d) Deep head of flexor pollicis brevis is innervated by ulna nerve

380) A

381) F F T

382) T T T F F

e) Abductor pollicis longus is innervated by posterior interosseous nerve

383) B

384) A

385) E

***386)* T T F F T**

387) Review A

The axillary nerve can be damaged in a shoulder dislocation or in a surgical neck fracture.

Clinical features-

•Abduction of the shoulder from 15 to 90 degrees is lost.

•Wasting of the deltoid.

•Loss of sensation over the lower half of the deltoid (policeman's batch sign)

388) Review C

Nipple retraction- infiltration of lactiferous ducts

Dumpling/retraction of the skin- malignant infiltration and fibrous contraction of Cooper's ligaments.

Peau d' orange appearance - obstruction of superficial lymphatics causing oedema

389) Review C

Features- paraesthesia, numbness over the distribution of the median nerve. The ulnar nerve and palmar cutaneous branch of the median nerve runs superficial to the flexor retinaculum. Therefore, they are not affected. No wasting of hypothenar eminence, no sensory loss over the thenar eminence, and no numbness over the medial one-and-a-half fingers.

390) Review C

Volkmann's ischemic contracture is due to ischemia followed by subsequent fibrosis and contraction of long flexors and extensors of the forearm. Flexors are more affected because they are bulkier than the extensors. This can be due to damage

to the brachial artery. Flexion at the wrist, extension at the metacarpophalangeal joints, and flexion at the interphalangeal joints occur.

391) Answer FFTTT

a)False-In erb's palsy upper trunk of the affected(C5,C6).
Arm is adducted and medially rotated,forearm extended and pronated. suprascapular,subscapular,musculocutaneuos, axillary nerves get damaged.
b)False
c) True
d)True
e)True

392) Answer-A

In this patient sensation over deltoid is lost.Axillary nerve supply sensory innervation to the slin over lower half of deltoid.(kuppi note 30)

393) Answer- B

The nerve that most prone to get damaged in a supracondylar fracture is median nerve.

394) Answer-E

Due to the damage of reccurent branch of the median nerve flexor pollicis brevis,oppones pollicis,abductor pollicis brevis muscles are paralyzed.But flexion at all joints of thumb is intact due to the action of flexor pollicis longus muscle.Froment's test is use to identification of ulnar nerve injuries.pen touch test is possitive when opponece pollicis muscle is paralyzed.That can be used to determine whether there is an damage to the reccurent branch of median nerve.

395) Answer-B

Dorsal interossei abduct-DAB
Palmar interossei adduct-PAD
Abduction of fingers exept middle finger affected.Thumb also can be abduct by abductor pollicis brevis.Therefore abduction of the ring finger affected more than the thumb.